

#1 . Class II Amalgam Prep

On the NDECC Class II amalgam preparation, what defines Competent for the proximal and gingival margins in Outline Form?

Quick Reference

The proximal and gingival margins clear the adjacent tooth by 0.5mm, with optimal buccal-lingual width and optimal reverse curve placement.

#2 . Class II Amalgam Prep

What proximal clearance range is Minimally Competent on the Class II amalgam prep Outline Form criterion?

Quick Reference

Proximal clearance greater than 0.5mm and less than 1.5mm, or greater than 0mm and less than 0.5mm, is Minimally Competent.

#3 . Class II Amalgam Prep

What gingival clearance range is Minimally Competent for Outline Form on the Class II amalgam prep?

Quick Reference

Gingival clearance greater than 0.5mm and less than 2.0mm is Minimally Competent.

#4 . Class II Amalgam Prep

At what proximal or gingival clearance does the Class II amalgam prep Outline Form become a Fail?

Quick Reference

A Fail occurs when proximal or gingival clearance is 1.5mm or greater.

#5 . Class II Amalgam Prep

Besides clearance, name three other ways Outline Form can Fail on the Class II amalgam prep.

Quick Reference

A prep too narrow for the smallest condenser, removal of sound tooth structure that compromises integrity, and divergent or flared proximal box walls are all Outline Form Fails.

#6 . Class II Amalgam Prep

What is the Competent pulpal floor depth range in Internal Form on the Class II amalgam prep?

Quick Reference

1.5mm to 2.0mm is the Competent pulpal floor depth.

#7 . Class II Amalgam Prep

What is the Competent mesial-distal gingival floor width on the Class II amalgam prep?

Quick Reference

1.5mm to 2.0mm mesial-distal gingival floor width is Competent.

#8 . Class II Amalgam Prep

What pulpal floor depth range is Minimally Competent on the Class II amalgam prep?

Quick Reference

A pulpal floor that is too shallow at 1.0mm to less than 1.5mm, or too deep at greater than 2.0mm to less than 3.0mm, is Minimally Competent.

#9 . Class II Amalgam Prep

At what pulpal floor depth does the Class II amalgam prep Internal Form Fail?

Quick Reference

Fail occurs at a pulpal floor depth less than 1.0mm or 3.0mm or greater.

#10 . Class II Amalgam Prep

What gingival floor width range is Minimally Competent for Internal Form on the amalgam prep?

Quick Reference

A gingival floor width greater than 2.0mm and less than 3.0mm is Minimally Competent.

#11 . Class II Amalgam Prep

At what gingival floor width does the Class II amalgam prep Internal Form Fail?

Quick Reference

Fail occurs at a gingival floor width less than 1.0mm or 3.0mm or greater.

#12 . Class II Amalgam Prep

Besides the depth and width thresholds, what other findings Fail the Internal Form criterion?

Quick Reference

Overpreparation causing unsupported enamel occlusally, and occlusally divergent internal form that compromises retention, are also Internal Form Fails.

#13 . Class II Amalgam Prep

What does Competent Internal Form require regarding internal line angles and resistance/retention form?

Quick Reference

Rounded internal line angles plus optimal resistance form and retention form.

#14 . Class II Amalgam Prep

What is Minimally Competent for resistance form and retention form on the amalgam prep?

Quick Reference

Adequate, though not optimal, resistance form and retention form.

#15 . Class II Amalgam Prep

What defines Competent Finish on the Class II amalgam preparation?

Quick Reference

Smooth cavosurface margins and internal surfaces with no unsupported enamel.

#16 . Class II Amalgam Prep

What is Minimally Competent for the Finish criterion on the amalgam prep?

Quick Reference

Unsupported enamel or roughness that does not compromise marginal integrity, and internal roughness that does not compromise restoration integrity.

#17 . Class II Amalgam Prep

When does the Finish criterion Fail on the Class II amalgam prep?

Quick Reference

Fail occurs when unsupported enamel or roughness compromises marginal integrity, or internal roughness compromises restoration integrity.

#18 . Class II Amalgam Prep

What is a cavosurface margin?

Quick Reference

The junction where the prepared internal walls of the cavity meet the external, unprepared surface of the tooth. Its smoothness is graded under the Finish criterion.

#19 . Class II Amalgam Prep

What defines Competent Damage on the Class II amalgam prep?

Quick Reference

No damage to the exam tooth beyond the prep itself, no damage to adjacent teeth, and no damage to soft tissue.

#20 . Class II Amalgam Prep

What is Minimally Competent for the Damage criterion?

Quick Reference

Damage correctable by enameloplasty, or minor soft tissue damage.

#21 . Class II Amalgam Prep

What findings Fail the Damage criterion on the amalgam prep?

Quick Reference

Damage requiring a restoration, excessive soft tissue damage, or tooth structure repaired with restorative material.

#22 . Class II Amalgam Prep

What is enameloplasty in the context of Damage grading?

Quick Reference

Reshaping or smoothing enamel to correct a minor defect without placing restorative material; damage correctable this way is graded Minimally Competent, not a Fail.

#23 . Class II Amalgam Prep

What are the Critical Errors for the Class II amalgam preparation?

Quick Reference

Any error that prevents evaluation of the exam tooth, or no preparation being performed on the assigned tooth. Either is an automatic fail regardless of other criteria.

#24 . Class II Amalgam Prep

What is a condenser, and how does it relate to Outline Form grading?

Quick Reference

A condenser is a hand instrument used to pack and compact amalgam into a cavity preparation. If the prep is too narrow for the smallest condenser to reach and compact the material, Outline Form is a Fail.

#25 . Class II Amalgam Prep

What is resistance form?

Quick Reference

The shape features of a preparation, such as a flat pulpal floor and rounded internal line angles, that help the tooth and the restoration withstand occlusal forces without fracturing.

#26 . Class II Amalgam Prep

What is retention form?

Quick Reference

The shape features of a preparation, such as parallel or slightly converging walls, that resist displacement of the restoration under lateral or dislodging forces.

#27 . Class II Amalgam Prep

What is the reverse curve in a Class II amalgam preparation, and which criterion grades it?

Quick Reference

A curved extension of the isthmus and pulpal floor that follows the natural marginal ridge contour rather than cutting straight across; its placement is graded as part of Outline Form.

#28 . Class II Amalgam Prep

Why must proximal box walls not be divergent or flared on the amalgam prep?

Quick Reference

Divergent or flared proximal box walls fail to retain the restoration and are graded a Fail under Outline Form.

#29 . Class II Amalgam Prep

Which two internal dimensions on the Class II amalgam prep share the identical Competent range of 1.5mm to 2.0mm?

Quick Reference

Pulpal floor depth and mesial-distal gingival floor width both have a Competent range of 1.5mm to 2.0mm.

#30 . Class II Amalgam Prep

How many grading criteria categories does the Class II amalgam preparation use?

Quick Reference

Four: Outline Form, Internal Form, Finish, and Damage, plus a separate list of Critical Errors.

#31 . Class II Amalgam Prep

On the NDECC, what are the three possible tiers for every grading criterion?

Quick Reference

Competent (pass, optimal), Minimally Competent (pass, acceptable), and Not Competent (fail).

#32 . Class II Amalgam Prep

Does the amalgam preparation criterion allow partial credit toward a passing grade?

Quick Reference

No. Like all seven Day 1 Clinical Skills procedures, it is graded Pass or Fail per procedure with no partial credit and no percentage score.

#33 . Class II Amalgam Prep

**Is proximal clearance of exactly 0.5mm
Competent or Minimally Competent?**

Quick Reference

Exactly 0.5mm is Competent. The margin must clear the adjacent tooth by 0.5mm to be Competent; Minimally Competent covers the ranges above and below that value up to 1.5mm.

#34 . Class II Amalgam Prep

**Is a gingival clearance of exactly 2.0mm a
Fail on the amalgam prep?**

Quick Reference

Yes. Fail is defined as gingival (or proximal) clearance of 1.5mm or greater, and 2.0mm is well past that threshold, so it fails outright rather than sitting in the Minimally Competent band, which only covers 0.5 to just under 2.0mm.

#35 . Class II Amalgam Prep

Is a pulpal floor depth of exactly 3.0mm a Fail?

Quick Reference

Yes, 3.0mm or greater is a Fail under Internal Form.

#36 . Class II Amalgam Prep

Is a mesial-distal gingival floor width of 0.9mm Competent, Minimally Competent, or Fail?

Quick Reference

Fail. Gingival floor width less than 1.0mm is a Fail, and 0.9mm is clearly below that threshold, well short of the Competent range of 1.5 to 2.0mm.

#37 . Class II Amalgam Prep

For the Class II amalgam preparation, on which tooth notation system are exam teeth identified, for example tooth 4.6?

Quick Reference

The FDI two-digit tooth notation (ISO 3950), used throughout the NDECC, not the American Universal Numbering System.

#38 . Class II Amalgam Prep

What does the Infection Control and Safety (IC&S;) allowance mean for a candidate performing the amalgam prep?

Quick Reference

IC&S; is assessed across the whole Day 1 exam, not per procedure. A candidate may make up to 3 IC&S; errors across the day; a 4th error is an automatic fail of the entire day.

#39 . Class II Amalgam Prep

Which single Class II amalgam prep criterion combines both a numeric clearance rule and a physical instrument-access rule?

Quick Reference

Outline Form: it sets numeric proximal and gingival clearance thresholds and also fails a prep too narrow for the smallest condenser to enter.

#40 . Class II Amalgam Prep

Summarize the four Class II amalgam prep criteria and their Critical Errors in one line each.

Quick Reference

Outline Form grades margin clearance and proximal box wall angulation; Internal Form grades pulpal floor depth, gingival floor width, and resistance/retention form; Finish grades smoothness of cavosurface margins and internal surfaces; Damage grades harm to the tooth, adjacent teeth, and soft tissue; Critical Errors are anything preventing evaluation or no preparation on the assigned tooth, either an automatic fail.

#41 . Class II Amalgam Restoration

For a Class II amalgam restoration, what defines a Competent margin?

Quick Reference

The tooth-restoration junction is not detectable. There is no palpable or visible step, ledge, or gap between the amalgam and the surrounding tooth structure.

#42 . Class II Amalgam Restoration

How much amalgam beyond the prep at the margin is still Minimally Competent?

Quick Reference

Amalgam extending up to and including 0.5mm beyond the prep is Minimally Competent, along with a margin deficiency correctable by enameloplasty.

#43 . Class II Amalgam Restoration

At what point does excess amalgam at the margin become a Fail?

Quick Reference

Any amalgam extending more than 0.5mm beyond the prep is a Fail, as is any margin deficiency or void that would require replacement of the restoration.

#44 . Class II Amalgam Restoration

What is the exact grading threshold that separates Minimally Competent from Fail for margin overhang on a Class II amalgam?

Quick Reference

0.5mm. Amalgam extending up to 0.5mm beyond the prep passes as Minimally Competent; anything greater than 0.5mm is a Fail.

#45 . Class II Amalgam Restoration

Define proximal contours as graded on a Class II amalgam restoration.

Quick Reference

Proximal contours describe how well the restoration reproduces the natural convex shape of the tooth's mesial or distal surface below the contact area, including the gingival embrasure form.

#46 . Class II Amalgam Restoration

What is a Competent result for proximal contours on a Class II amalgam?

Quick Reference

Physiologic contours are optimally restored and the proximal contact is optimal.

#47 . Class II Amalgam Restoration

What under/overcontour amount at the axial surface is Minimally Competent for proximal contours?

Quick Reference

Under or overcontour that is less than 1.0mm and limited to the axial surface only is Minimally Competent.

#48 . Class II Amalgam Restoration

What is the exact grading threshold for axial surface contour deviation on a Class II amalgam proximal contour?

Quick Reference

<1.0mm. Axial surface under or overcontour of less than 1.0mm is Minimally Competent; the reference table gives no explicit upper Fail number, but a deviation that negatively impacts gingival health is a Fail regardless of magnitude.

#49 . Class II Amalgam Restoration

What proximal contact quality is Minimally Competent rather than Competent?

Quick Reference

A contact that is present but small, light, or not optimal is Minimally Competent, since it still functions as a contact even though it falls short of the ideal.

#50 . Class II Amalgam Restoration

What two proximal contact conditions are graded as a Fail?

Quick Reference

An absent proximal contact where floss passes through with no resistance, or a contact so tight it blocks floss entirely, are both Fail.

#51 . Class II Amalgam Restoration

What over/undercontour condition of the proximal surface is a Fail?

Quick Reference

Over or undercontour that negatively impacts gingival health is a Fail, regardless of the millimetre measurement.

#52 . Class II Amalgam Restoration

Define the term physiologic contour.

Quick Reference

Physiologic contour is the natural, healthy convex shape of a tooth surface that supports proper gingival contour, food deflection away from the gingiva, and periodontal health.

#53 . Class II Amalgam Restoration

Define the term proximal contact.

Quick Reference

Proximal contact is the point where the mesial or distal surface of a restored tooth touches the adjacent tooth, holding the arch together and preventing food impaction interproximally.

#54 . Class II Amalgam Restoration

How is proximal contact quality clinically tested?

Quick Reference

Dental floss is passed through the contact. A correct contact allows floss through with some resistance (a slight snap); no resistance means the contact is absent (Fail), and floss that cannot pass at all means the contact is too tight (Fail).

#55 . Class II Amalgam Restoration

What does the occlusal morphology and function criterion evaluate on a Class II amalgam?

Quick Reference

It evaluates whether the occlusal surface reproduces natural anatomic form (cusps, ridges, grooves) and whether the resulting occlusal contacts function properly with the opposing dentition.

#56 . Class II Amalgam Restoration

What is a Competent result for occlusal morphology and function?

Quick Reference

Optimal occlusal contacts and morphology that approximates natural anatomy.

#57 . Class II Amalgam Restoration

What over/undercontour condition of occlusal morphology is Minimally Competent?

Quick Reference

Over or undercontour that does not affect the occlusal contacts is Minimally Competent, since function is preserved even though anatomic form is imperfect.

#58 . Class II Amalgam Restoration

What supraocclusion finding is Minimally Competent for a Class II amalgam?

Quick Reference

Supraocclusion (the restoration is high) is Minimally Competent only if light contacts are still present elsewhere in the quadrant, meaning the bite is not fully thrown off.

#59 . Class II Amalgam Restoration

When is supraocclusion a Fail rather than Minimally Competent?

Quick Reference

Supraocclusion is a Fail when it risks fracture or trauma, meaning the restoration is so high it concentrates destructive occlusal force.

#60 . Class II Amalgam Restoration

What undercontoured occlusal finding is a Fail on a Class II amalgam?

Quick Reference

An undercontoured occlusal surface that is too thin and risks fracture is a Fail.

#61 . Class II Amalgam Restoration

Define the term articulating paper and its use in grading a Class II amalgam.

Quick Reference

Articulating paper is thin coloured marking paper placed between the teeth to mark occlusal contact points. It is used to verify occlusal contacts are present, appropriately distributed, and not supraocclusal on the new restoration.

#62 . Class II Amalgam Restoration

What is a Competent result for the finish criterion on a Class II amalgam?

Quick Reference

Optimal surface quality: the restoration is smooth with no roughness or voids affecting contours or margins.

#63 . Class II Amalgam Restoration

What roughness finding is Minimally Competent for finish on a Class II amalgam?

Quick Reference

Roughness that is correctable by polishing without impacting the contours of the restoration is Minimally Competent.

#64 . Class II Amalgam Restoration

What roughness finding is a Fail for finish on a Class II amalgam?

Quick Reference

Roughness that impacts the contours (creating a plaque retention risk) is a Fail.

#65 . Class II Amalgam Restoration

Besides roughness, what other finish finding is automatically a Fail on a Class II amalgam?

Quick Reference

Voids anywhere other than at the margin that require replacement of the restoration are a Fail.

#66 . Class II Amalgam Restoration

Define the term burnishing as it relates to amalgam restoration.

Quick Reference

Burnishing is rubbing the surface of a condensed or carved amalgam restoration with a smooth instrument to compact and smooth the surface, improve marginal adaptation, and reduce voids before final carving and polishing.

#67 . Class II Amalgam Restoration

Define the term condensation as it relates to amalgam restoration.

Quick Reference

Condensation is the process of packing freshly triturated amalgam into the prepared cavity in increments using pressure, which removes excess mercury, eliminates voids, and adapts the material tightly against the cavity walls and margins.

#68 . Class II Amalgam Restoration

What damage tiers apply to the exam tooth, adjacent teeth, or soft tissue on a Class II amalgam restoration?

Quick Reference

Competent is no damage at all. Minimally Competent is damage correctable by enameloplasty or minor soft tissue damage. Fail is damage requiring a restoration.

#69 . Class II Amalgam Restoration

What defines Minimally Competent damage on a Class II amalgam restoration?

Quick Reference

Damage that is correctable by enameloplasty, or minor soft tissue damage, is Minimally Competent.

#70 . Class II Amalgam Restoration

What defines a Fail level of damage on a Class II amalgam restoration?

Quick Reference

Damage requiring a restoration is a Fail, meaning the harm to the exam tooth, an adjacent tooth, or soft tissue is severe enough that a simple correction is not sufficient.

#71 . Class II Amalgam Restoration

List the critical errors defined for a Class II amalgam restoration.

Quick Reference

Anything that prevents evaluation, foreign objects or materials remaining in the oral cavity, no restoration placed, wrong tooth restored, inappropriate or incorrect material used, or the restoration fractured or loose.

#72 . Class II Amalgam Restoration

Why is 'wrong tooth restored' a critical error rather than just a Fail on any single criterion?

Quick Reference

A critical error automatically fails the entire procedure regardless of how well the actual restoration was executed, because it represents a patient safety or scope-of-practice breach rather than a quality deficiency.

#73 . Class II Amalgam Restoration

Why does 'foreign objects or materials remaining in the oral cavity' count as a critical error?

Quick Reference

Retained foreign material poses an immediate risk to the patient (aspiration, soft tissue injury, infection) and is treated as an automatic critical error independent of the restoration's quality.

#74 . Class II Amalgam Restoration

If a Class II amalgam restoration is fractured or loose at evaluation, what grading outcome applies?

Quick Reference

It is a critical error, which fails the procedure outright regardless of how the margin, contours, morphology, and finish would otherwise have graded.

#75 . Class II Amalgam Restoration

Compare the margin overhang threshold for Class II amalgam restoration to Class II composite resin restoration.

Quick Reference

Both share the same Margin / Proximal Contours / Occlusal Morphology and Function / Finish / Damage structure and the same 0.5mm margin threshold; composite restoration adds material-specific finish failures such as incomplete polymerization and contamination requiring replacement.

#76 . Class II Amalgam Restoration

How does the amalgam restoration finish criterion differ from the composite resin restoration finish criterion?

Quick Reference

Amalgam finish is graded on smoothness, roughness, and non-margin voids only. Composite resin restoration finish adds incomplete polymerization as a Fail and grades surface contamination (Minimally Competent if correctable, Fail if it requires full replacement).

#77 . Class II Amalgam Restoration

A Class II amalgam has a proximal contact that a floss strand passes through with a light snap and moderate resistance. How is this graded?

Quick Reference

Competent, since an optimal proximal contact allows floss through with resistance rather than passing freely or being blocked entirely.

#78 . Class II Amalgam Restoration

A Class II amalgam restoration is 0.7mm proud of the prep margin on the buccal surface. How is this graded?

Quick Reference

Fail, because any amalgam extending more than 0.5mm beyond the prep at the margin fails, and 0.7mm exceeds that 0.5mm threshold.

#79 . Class II Amalgam Restoration

A Class II amalgam on tooth 3.6 shows occlusal contacts confirmed by articulating paper marks that are heavier than the adjacent natural teeth in the same quadrant, with no other quadrant contacts visible. How is this graded?

Quick Reference

Fail, since the restoration is supraocclusal with no light contacts remaining elsewhere in the quadrant, risking fracture or trauma rather than qualifying as the Minimally Competent supraocclusion allowance.

#80 . Class II Amalgam Restoration

A Class II amalgam restoration on tooth 4.6 has an undercontoured axial proximal surface measuring 0.6mm below the ideal contour, with no impact on gingival health. How is this graded?

Quick Reference

Minimally Competent, since it is an axial-surface-only under/overcontour of less than 1.0mm and does not negatively affect gingival health.

#81 . Class II Composite Restoration

What five criteria categories is the Class II composite resin restoration graded on?

Quick Reference

Margin, Proximal Contours, Occlusal Morphology and Function, Finish, and Damage, the same structure used for the Class II amalgam restoration.

#82 . Class II Composite Restoration

What is the Competent standard for the Margin criterion on a Class II composite restoration?

Quick Reference

The tooth-restoration junction is not detectable.

#83 . Class II Composite Restoration

What Margin finding is graded Minimally Competent on a Class II composite restoration?

Quick Reference

A deficiency correctable by enameloplasty, or composite extending no more than 0.5mm beyond the preparation.

#84 . Class II Composite Restoration

What Margin finding is a Fail on a Class II composite restoration?

Quick Reference

A deficiency or void requiring replacement, or composite extending more than 0.5mm beyond the preparation.

#85 . Class II Composite Restoration

What is the composite-specific Finish rule for polymerization on a Class II composite restoration?

Quick Reference

Incomplete polymerization is a Fail. There is no minimally competent tier for this item.

#86 . Class II Composite Restoration

How are internal or surface voids and porosities graded under Finish for a Class II composite restoration?

Quick Reference

Voids or porosities that compromise esthetics or durability are a Fail.

#87 . Class II Composite Restoration

How is contamination of the composite restoration graded under Finish?

Quick Reference

Contamination requiring correction is Minimally Competent. Contamination requiring full replacement of the restoration is a Fail.

#88 . Class II Composite Restoration

What is the Competent standard for Finish on a Class II composite restoration, aside from the composite-specific items?

Quick Reference

Optimal surface quality with a smooth, well-polished result and no contamination or voids.

#89 . Class II Composite Restoration

What roughness finding is Minimally Competent under Finish?

Quick Reference

Roughness correctable by polishing without impacting the restoration's contours.

#90 . Class II Composite Restoration

What roughness finding is a Fail under Finish?

Quick Reference

Roughness impacting the contours, which creates a plaque retention risk.

#91 . Class II Composite Restoration

What is the Competent standard for Proximal Contours on a Class II composite restoration?

Quick Reference

Physiologic contours are optimally restored with an optimal proximal contact.

#92 . Class II Composite Restoration

What proximal contour finding is Minimally Competent?

Quick Reference

Under- or overcontour of less than 1.0mm on the axial surface only, or a contact that is present but small, light, or not optimal.

#93 . Class II Composite Restoration

What two proximal contour findings are automatic Fails?

Quick Reference

Over- or undercontour negatively impacting gingival health, or an absent proximal contact where floss passes with no resistance or a contact that blocks floss entirely.

#94 . Class II Composite Restoration

What is the Competent standard for Occlusal Morphology and Function?

Quick Reference

Optimal occlusal contacts and morphology that approximate natural anatomy.

#95 . Class II Composite Restoration

What occlusal finding is Minimally Competent?

Quick Reference

Over- or undercontour that does not affect occlusal contacts, or supraocclusion with only light contacts elsewhere in the quadrant.

#96 . Class II Composite Restoration

What two occlusal findings are automatic Fails?

Quick Reference

Supraocclusion that risks fracture or trauma, or an undercontoured occlusal surface that is too thin and risks fracture.

#97 . Class II Composite Restoration

What is the Competent standard for Damage on a Class II composite restoration?

Quick Reference

No damage to the exam tooth beyond the restoration, to adjacent teeth, or to soft tissue.

#98 . Class II Composite Restoration

What damage findings are Minimally Competent?

Quick Reference

Damage correctable by enameloplasty, or minor soft tissue damage.

#99 . Class II Composite Restoration

What damage findings are automatic Fails?

Quick Reference

Damage requiring a restoration, excessive soft tissue damage, or tooth structure repaired with restorative material.

#100 . Class II Composite Restoration

List the Critical Errors for a Class II composite restoration.

Quick Reference

Any error preventing evaluation of the exam tooth, foreign objects or materials remaining in the oral cavity, no restoration placed, wrong tooth restored, inappropriate or incorrect material used, and a restoration that is fractured or loose.

#101 . Class II Composite Restoration

Is a fractured or loose composite restoration graded as a Fail criterion or a Critical Error?

Quick Reference

It is a Critical Error, meaning an automatic fail of the procedure regardless of the scores on other criteria.

#102 . Class II Composite Restoration

Why does incomplete polymerization carry no Minimally Competent tier, unlike most other Finish items?

Quick Reference

Incomplete polymerization means the composite has not fully hardened, so the material properties and clinical durability of the restoration cannot be considered adequate at any level, making it a straight Fail.

#103 . Class II Composite Restoration

Define incremental layering in the context of Class II composite placement.

Quick Reference

Placing and light-curing the composite in small increments, generally no more than about 2mm thick, rather than bulk-filling the cavity in one step.

#104 . Class II Composite Restoration

Why is incremental layering used for Class II composite restorations?

Quick Reference

It reduces the volume of material curing at once, which lowers polymerization shrinkage stress per increment and improves depth of cure and marginal adaptation.

#105 . Class II Composite Restoration

Define polymerization shrinkage.

Quick Reference

The volumetric contraction that composite resin undergoes as it hardens during light-curing, which can generate stress at the tooth-restoration interface.

#106 . Class II Composite Restoration

What clinical problems can excessive polymerization shrinkage stress cause?

Quick Reference

Marginal gap formation, microleakage, postoperative sensitivity, and cuspal flexure, all of which can contribute to a Margin or Finish deficiency.

#107 . Class II Composite Restoration

How does incremental layering help control polymerization shrinkage compared to a single bulk increment?

Quick Reference

Each smaller increment shrinks toward a smaller bonded surface area, reducing the configuration factor and resulting stress, and each increment is more fully cured before the next is added.

#108 . Class II Composite Restoration

What is a matrix band used for in a Class II composite restoration?

Quick Reference

A thin metal or clear plastic strip placed around the tooth to form the missing proximal wall, allowing the composite to be contoured and contained during placement.

#109 . Class II Composite Restoration

Why might a clear matrix band be preferred over a metal band for an esthetic composite restoration?

Quick Reference

A clear band allows light transmission through the band, helping light-curing reach and polymerize composite near the proximal margin.

#110 . Class II Composite Restoration

What is a wedge and why is it placed with the matrix band?

Quick Reference

A wedge is a small triangular device inserted interproximally at the gingival margin to adapt the matrix band tightly against the tooth, preventing an overhang and helping re-establish proximal contact.

#111 . Class II Composite Restoration

What Margin or Finish problem can result from an inadequately adapted matrix band or wedge?

Quick Reference

A composite overhang or void at the gingival margin, which can be graded as a Margin deficiency or, if severe, a Fail requiring replacement.

#112 . Class II Composite Restoration

What does isolation mean in the context of a Class II composite restoration?

Quick Reference

Keeping the prepared tooth free of saliva, blood, and other moisture contamination during bonding and composite placement, typically with a rubber dam or effective cotton roll and suction technique.

#113 . Class II Composite Restoration

Why is isolation especially critical for composite restorations compared to amalgam?

Quick Reference

Composite bonds adhesively to tooth structure, so moisture contamination during etching, bonding, or placement can compromise the bond and lead to a Finish contamination finding.

#114 . Class II Composite Restoration

If saliva contaminates the bonded surface partway through a Class II composite restoration, how would correcting it versus not correcting it be graded?

Quick Reference

If the contamination is corrected appropriately, it is graded Minimally Competent under Finish. If it requires full replacement of the restoration, it is a Fail.

#115 . Class II Composite Restoration

Define light-curing in the context of composite restoration.

Quick Reference

The process of applying a curing light of the appropriate wavelength and intensity to initiate and complete polymerization of a light-cured composite resin increment.

#116 . Class II Composite Restoration

Define depth of cure.

Quick Reference

The maximum thickness of a composite increment that a curing light can adequately polymerize, beyond which the composite deeper in the increment remains undercured.

#117 . Class II Composite Restoration

How does exceeding the depth of cure relate to the Finish grading for polymerization?

Quick Reference

Placing an increment thicker than the depth of cure can leave the deeper composite incompletely polymerized, which is graded as a Fail under Finish.

#118 . Class II Composite Restoration

Why should the curing light tip be held as close as possible to the composite surface during light-curing?

Quick Reference

Light intensity decreases with distance from the tip, so increasing the distance reduces the effective depth of cure and raises the risk of incomplete polymerization.

#119 . Class II Composite Restoration

What is the same across the Margin, Proximal Contours, Occlusal Morphology and Function, and Damage criteria for Class II composite versus Class II amalgam restorations?

Quick Reference

The grading tiers and thresholds for these four criteria are identical between the two restoration types; only the Finish criterion adds composite-specific items for polymerization, voids and porosities, and contamination.

#120 . Class II Composite Restoration

What are the Critical Errors shared between the Class II composite restoration and the Class II amalgam restoration?

Quick Reference

Any error preventing evaluation, foreign objects or materials remaining in the oral cavity, no restoration placed, wrong tooth restored, inappropriate or incorrect material used, and a restoration that is fractured or loose.

#121 . Class IV Composite Restoration

What tooth is restored in the Class IV composite resin restoration procedure on Day 1 of the NDECC?

Quick Reference

An anterior tooth. Because it is in the esthetic zone, esthetics is explicitly assessed in the grading criteria, unlike the posterior Class II restorations.

#122 . Class IV Composite Restoration

Which five criteria categories is the Class IV composite resin restoration graded on?

Quick Reference

Margin, Proximal Contours, Occlusion and Function, Finish, and Damage.

#123 . Class IV Composite Restoration

What is the Competent standard for the Margin criterion on a Class IV composite restoration?

Quick Reference

The tooth-restoration junction is not detectable, meaning the composite blends seamlessly with the surrounding tooth structure at the margin.

#124 . Class IV Composite Restoration

What makes a Class IV margin Minimally Competent?

Quick Reference

A deficiency that is correctable by enameloplasty, or composite extending no more than 0.5mm beyond the preparation.

#125 . Class IV Composite Restoration

What makes a Class IV margin a Fail?

Quick Reference

A deficiency or void requiring replacement, or composite extending more than 0.5mm beyond the preparation.

#126 . Class IV Composite Restoration

What is the Competent standard for Proximal Contours on a Class IV composite restoration?

Quick Reference

Physiologic contours are optimally restored with an optimal proximal contact.

#127 . Class IV Composite Restoration

How does the Minimally Competent standard for Proximal Contours differ between Class II and Class IV restorations?

Quick Reference

Class II allows an undercontour or overcontour under 1.0mm on the axial surface only. Class IV instead requires the contour not reproduce natural anatomical contour but must NOT impact gingival health or esthetics, reflecting the anterior esthetic zone.

#128 . Class IV Composite Restoration

What makes Proximal Contours a Fail on a Class IV composite restoration?

Quick Reference

Over or undercontour negatively impacting gingival health OR esthetics, or an absent proximal contact where floss passes with no resistance, or a contact that blocks floss entirely.

#129 . Class IV Composite Restoration

Why does the Class IV Proximal Contours fail criterion explicitly include esthetics while Class II does not?

Quick Reference

Class IV restores an anterior tooth in the esthetic zone, so a contour flaw that harms appearance is graded as a fail even if gingival health is unaffected, unlike the posterior Class II restoration.

#130 . Class IV Composite Restoration

What esthetic-specific fail language applies to the incisal edge on a Class IV composite restoration?

Quick Reference

Incisal edge disharmony negatively impacting esthetics is listed as a Fail.

#131 . Class IV Composite Restoration

What does incisal edge disharmony mean in the context of a Class IV restoration?

Quick Reference

A mismatch in the shape, length, or alignment of the restored incisal edge relative to the natural tooth or adjacent teeth that disrupts the appearance of the smile.

#132 . Class IV Composite Restoration

What esthetic-specific fail language applies to the labial or lingual surface on a Class IV composite restoration?

Quick Reference

Overcontour or undercontour of the labial or lingual surface not reproducing natural anatomical contour, negatively impacting esthetics or function, is listed as a Fail.

#133 . Class IV Composite Restoration

Which surface faces the lips on an anterior tooth restored with a Class IV composite?

Quick Reference

The labial surface.

#134 . Class IV Composite Restoration

Which surface faces the tongue on an anterior tooth restored with a Class IV composite?

Quick Reference

The lingual surface.

#135 . Class IV Composite Restoration

What is the Competent standard for Occlusion and Function on a Class IV composite restoration?

Quick Reference

Optimal occlusal contacts and morphology approximating natural anatomy, consistent with the Class II occlusal standard applied to the anterior restoration's function.

#136 . Class IV Composite Restoration

What is a Fail for Finish on a Class IV composite restoration related to polymerization?

Quick Reference

Incomplete polymerization is a Fail, the same composite-specific finish standard used for Class II composite restorations.

#137 . Class IV Composite Restoration

What is the Fail standard for voids or porosities in the Finish criterion of a Class IV composite restoration?

Quick Reference

Internal or surface voids or porosities compromising esthetics or durability are a Fail.

#138 . Class IV Composite Restoration

How is contamination graded under Finish for a Class IV composite restoration?

Quick Reference

Contamination requiring correction is Minimally Competent. Contamination requiring full replacement of the restoration is a Fail.

#139 . Class IV Composite Restoration

What is the Competent standard for Damage on a Class IV composite restoration?

Quick Reference

No damage to the exam tooth beyond the preparation, to adjacent teeth, or to soft tissue.

#140 . Class IV Composite Restoration

What Damage findings are Minimally Competent on a Class IV composite restoration?

Quick Reference

Damage correctable by enameloplasty, or minor soft tissue damage.

#141 . Class IV Composite Restoration

What Damage findings are a Fail on a Class IV composite restoration?

Quick Reference

Damage requiring a restoration, excessive soft tissue damage, or tooth structure repaired with restorative material.

#142 . Class IV Composite Restoration

What are the Critical Errors for the Class IV composite resin restoration procedure?

Quick Reference

Same as the other restoration procedures: any error preventing evaluation of the exam tooth, foreign objects or materials remaining in the oral cavity, no restoration placed, wrong tooth restored, inappropriate or incorrect material used, or restoration fractured or loose.

#143 . Class IV Composite Restoration

Does a Critical Error on a Class IV composite restoration allow partial credit?

Quick Reference

No. Any Critical Error is an automatic fail for the procedure with no partial credit, the same rule that applies across all seven Day 1 clinical procedures.

#144 . Class IV Composite Restoration

What does shade selection mean when preparing a Class IV composite restoration?

Quick Reference

Choosing the composite resin colour, or combination of colours, that matches the surrounding natural tooth structure so the restoration blends in rather than appearing as a visible patch.

#145 . Class IV Composite Restoration

Why is shade selection especially important for a Class IV restoration but not graded for a Class II restoration on the NDECC?

Quick Reference

The Class IV restoration is on an anterior tooth in the esthetic zone where a visible colour mismatch is immediately noticeable, while the Class II restoration is on a posterior tooth where colour match matters less to visible esthetics.

#146 . Class IV Composite Restoration

What does translucency mean in the context of composite resin used for a Class IV restoration?

Quick Reference

The degree to which light passes through the material rather than being fully opaque or fully reflected, mimicking how natural enamel allows light to pass and gives teeth their lifelike depth.

#147 . Class IV Composite Restoration

Why does matching translucency matter for a Class IV composite restoration's esthetic outcome?

Quick Reference

Natural enamel is translucent at the incisal edge while dentine underneath is more opaque. A restoration with the wrong translucency looks flat, chalky, or too dark under different lighting compared to the surrounding natural tooth.

#148 . Class IV Composite Restoration

What is a line angle on a tooth being restored with Class IV composite?

Quick Reference

The junction line formed where two flat or curved tooth surfaces meet, such as where the labial surface meets the mesial or distal surface.

#149 . Class IV Composite Restoration

Why do line angles matter to the esthetic grading of a Class IV composite restoration?

Quick Reference

Natural anterior teeth have subtle, rounded line angles. Overcontouring or undercontouring the labial or lingual surface disrupts these line angles, which is graded as a Fail when it negatively impacts esthetics or function.

#150 . Class IV Composite Restoration

What is the incisal edge of an anterior tooth restored with Class IV composite?

Quick Reference

The biting edge of an anterior tooth, formed where the labial and lingual surfaces converge at the tip of the tooth.

#151 . Class IV Composite Restoration

What specific incisal edge finding is listed as a Fail for the Class IV composite restoration?

Quick Reference

Incisal edge disharmony negatively impacting esthetics, meaning the restored incisal edge does not match the natural tooth's or adjacent teeth's incisal outline in a way that disrupts appearance.

#152 . Class IV Composite Restoration

What is meant by the esthetic zone in restorative dentistry, as it applies to the Class IV composite restoration?

Quick Reference

The visible region of the smile, typically the anterior teeth, where appearance is a primary concern, unlike posterior teeth where function dominates over visible esthetics.

#153 . Class IV Composite Restoration

How does grading in the esthetic zone change the Fail threshold for proximal contours compared to a posterior restoration?

Quick Reference

In the esthetic zone, a contour that fails to reproduce natural anatomical form is a Fail if it negatively impacts esthetics alone, even without harming gingival health, unlike a posterior Class II restoration where only gingival health impact is named.

#154 . Class IV Composite Restoration

What is layering technique in composite resin restoration?

Quick Reference

Placing and curing the composite resin in successive thin increments, often using different shades or opacities for the dentine and enamel layers, rather than in a single bulk placement.

#155 . Class IV Composite Restoration

Why is layering technique particularly relevant to a Class IV composite restoration?

Quick Reference

Because it is an anterior restoration in the esthetic zone, layering allows the candidate to replicate natural tooth structure with a more opaque dentine layer covered by a translucent enamel layer, improving both esthetics and the chance of complete polymerization throughout the restoration.

#156 . Class IV Composite Restoration

What Class IV Finish criterion connects most directly to a poorly executed layering technique?

Quick Reference

Internal or surface voids or porosities compromising esthetics or durability, which is a Fail and can result from air entrapment or incomplete bonding between layers.

#157 . Class IV Composite Restoration

Which criterion category on the Class IV composite restoration would grade an open or overly tight proximal contact with an adjacent anterior tooth?

Quick Reference

Proximal Contours. An absent contact where floss passes with no resistance, or a contact that blocks floss entirely, is a Fail.

#158 . Class IV Composite Restoration

Under the Class IV composite restoration grading table, is a small, light, or not-optimal proximal contact a Fail?

Quick Reference

No. A contact present but small, light, or not optimal is Minimally Competent, not a Fail, matching the standard used for the other restoration procedures.

#159 . Class IV Composite Restoration

If a labial surface overcontour negatively impacts function but not esthetics, how is it graded on a Class IV composite restoration?

Quick Reference

It is still a Fail. The criterion reads negatively impacting esthetics OR function, so either negative impact alone is enough to fail that surface's contour.

#160 . Class IV Composite Restoration

Besides Margin, Proximal Contours, Occlusion and Function, Finish, and Damage, what other assessment applies throughout Day 1 during a Class IV composite restoration?

Quick Reference

Infection Control and Safety, IC&S;, which permits a maximum of three errors across the whole day, with a fourth error being an automatic fail regardless of the restoration's own grading.

#161 . Endodontic Access Prep

On NDECC Day 1, which criteria categories is the endodontic access cavity preparation graded against?

Quick Reference

It is one of 7 manikin procedures graded Pass/Fail with no partial credit, assessed on External Outline Form, Internal Form, Finish, and Damage, plus a separate list of Critical Errors.

#162 . Endodontic Access Prep

What are the four criteria categories for grading endodontic access cavity preparation?

Quick Reference

External Outline Form, Internal Form, Finish, and Damage.

#163 . Endodontic Access Prep

List the Critical Errors that cause an automatic fail on the endodontic access preparation, regardless of other scores.

Quick Reference

Any error preventing evaluation of the exam tooth, no preparation performed on the assigned tooth, and access initiated on the wrong tooth.

#164 . Endodontic Access Prep

If a candidate initiates the access cavity on the wrong tooth, is this correctable through the other criteria?

Quick Reference

No. Access initiated on the wrong tooth is a named Critical Error, an automatic fail regardless of how well the preparation is otherwise executed.

#165 . Endodontic Access Prep

Define straight-line access in the context of endodontic access cavity preparation.

Quick Reference

An outline and internal form that lets an instrument travel from the access opening directly into each canal orifice without deflection, avoiding obstruction from chamber walls, pulp horns, or the chamber roof.

#166 . Endodontic Access Prep

What does Competent External Outline Form require regarding straight-line access?

Quick Reference

Optimal extension of the outline for straight-line access to all canals, optimal removal of unsupported tooth structure, and complete removal of the pulp horns.

#167 . Endodontic Access Prep

What is Minimally Competent External Outline Form?

Quick Reference

The outline still allows access to all canals, or there is overextension that does not compromise structural integrity of the tooth.

#168 . Endodontic Access Prep

What three findings make External Outline Form a Fail?

Quick Reference

Pulp chamber not accessed, outline prevents access to any canal, or overextension beyond a cusp or marginal ridge.

#169 . Endodontic Access Prep

A candidate finishes the access cavity but the pulp chamber was never actually entered. What grade does External Outline Form receive?

Quick Reference

Fail. Pulp chamber not accessed is one of the three named Fail criteria for External Outline Form.

#170 . Endodontic Access Prep

The outline extends slightly beyond the marginal ridge but the candidate believes the tooth's structural integrity is intact. Is this Fail or Minimally Competent?

Quick Reference

Minimally Competent, provided the overextension truly does not compromise structural integrity. Overextension actually beyond a cusp or marginal ridge is Fail; the two tiers are separated by whether structural integrity is compromised.

#171 . Endodontic Access Prep

What are pulp horns and why must they be removed during access preparation?

Quick Reference

Pulp horns are the coronal projections of pulp tissue extending into the cusps beneath the pulp chamber roof. Leaving them unremoved obstructs straight-line access and separates Competent from Minimally Competent External Outline Form.

#172 . Endodontic Access Prep

To what depth must a canal be accessed for Competent Internal Form?

Quick Reference

Canals accessed to a depth of 2.0mm.

#173 . Endodontic Access Prep

What canal access depth defines Minimally Competent Internal Form?

Quick Reference

Canals accessed to a depth of less than 2.0mm, with pulp horns and the pulp chamber roof removed and no perforation.

#174 . Endodontic Access Prep

What three findings make Internal Form a Fail on the endodontic access preparation?

Quick Reference

Major obstruction to a canal, unnecessary removal of tooth structure that alters the restorative treatment plan, or perforation.

#175 . Endodontic Access Prep

A candidate accesses a canal to only 1.0mm depth with no perforation and no obstruction. What is the Internal Form grade?

Quick Reference

Minimally Competent. The depth is less than 2.0mm but there is no perforation and no major obstruction, so it does not fall to Fail.

#176 . Endodontic Access Prep

Define perforation as it relates to endodontic access preparation grading.

Quick Reference

An unintended hole created through the floor or wall of the pulp chamber or a canal wall during access preparation, connecting the chamber or canal to the periodontal tissues or external tooth surface. It is a named Fail criterion under Internal Form.

#177 . Endodontic Access Prep

Is minimal gouging of the chamber wall automatically a Fail?

Quick Reference

No. Minimal gouging is Minimally Competent as long as it does not impact the structural integrity of the tooth; gouging that compromises structural integrity or represents a major obstruction is Fail.

#178 . Endodontic Access Prep

What does Competent Finish require for the endodontic access preparation?

Quick Reference

Optimal smoothness of the preparation with no pulp material remaining on the walls or floor of the chamber.

#179 . Endodontic Access Prep

What is Minimally Competent Finish for the access cavity?

Quick Reference

Roughness that is correctable without impacting structural integrity, or minimal pulp material or debris remaining that still allows canal access.

#180 . Endodontic Access Prep

List the three Fail conditions under Finish for endodontic access preparation.

Quick Reference

Roughness that would negatively impact structural integrity if corrected, tooth structure rebuilt with restorative material, or excessive pulp material or debris obstructing the chamber or canal.

#181 . Endodontic Access Prep

If debris remains in the chamber but a canal orifice can still be located and entered, what is the Finish grade?

Quick Reference

Minimally Competent, since minimal pulp material or debris that still allows canal access does not fail the criterion; it becomes Fail only when debris is excessive enough to obstruct the chamber or canal.

#182 . Endodontic Access Prep

What does Competent Damage mean for the endodontic access preparation?

Quick Reference

No damage to the exam tooth beyond the intended preparation, no damage to adjacent teeth, and no damage to soft tissue.

#183 . Endodontic Access Prep

What is Minimally Competent Damage for this procedure?

Quick Reference

Damage correctable by enameloplasty, or minor soft tissue damage.

#184 . Endodontic Access Prep

List the Fail conditions under Damage for endodontic access preparation.

Quick Reference

Damage that alters the restorative treatment plan, damage requiring a restoration, excessive soft tissue damage, or the tooth repaired with restorative material.

#185 . Endodontic Access Prep

A candidate accidentally nicks an adjacent tooth during access preparation but the nick can be smoothed by enameloplasty. What Damage grade results?

Quick Reference

Minimally Competent, since damage correctable by enameloplasty does not fall to Fail; Fail requires damage that alters the treatment plan or requires an actual restoration.

#186 . Endodontic Access Prep

How many named Critical Errors apply specifically to endodontic access preparation grading?

Quick Reference

Three: any error preventing evaluation of the exam tooth, no preparation performed on the assigned tooth, and access initiated on the wrong tooth.

#187 . Endodontic Access Prep

What instrument is classically used to locate canal orifices during access preparation, and what is distinctive about its tip?

Quick Reference

The DG16 endodontic explorer, which has a sharp, rigid, sickle-shaped double-ended tip designed to catch and drop into the subtle depression of a canal orifice on the pulp chamber floor.

#188 . Endodontic Access Prep

Define the pulp chamber roof.

Quick Reference

The coronal-most wall of the pulp chamber, located beneath the occlusal or incisal surface, which must be fully removed to expose the pulp horns and floor and permit straight-line access.

#189 . Endodontic Access Prep

Define canal orifice.

Quick Reference

The opening on the floor of the pulp chamber where a root canal begins, typically located by tactile feel with a sharp explorer such as the DG16 and visually as a dark or lined depression against the chamber floor.

#190 . Endodontic Access Prep

Why is complete removal of the pulp chamber roof necessary for Competent grading, beyond simply improving visibility?

Quick Reference

An unremoved roof or remaining pulp horn tissue physically blocks the straight path from the access opening into the canal, which is exactly what Competent External Outline Form and Internal Form require.

#191 . Endodontic Access Prep

What outline shape is used for the access cavity on a maxillary or mandibular anterior tooth such as tooth 1.1 or tooth 4.1?

Quick Reference

A triangular outline on the lingual surface, with the base near the cingulum and the apex toward the incisal edge, following the single pulp horn arrangement of the pulp chamber.

#192 . Endodontic Access Prep

What outline shape is typical for the access cavity on a maxillary premolar such as tooth 1.4?

Quick Reference

An oval outline in the central fossa area, oriented buccolingually to reflect the two canal orifices, buccal and palatal, typically present.

#193 . Endodontic Access Prep

What outline shape is typical for the access cavity on a maxillary molar such as tooth 1.6?

Quick Reference

A triangular outline over the mesial marginal ridge area of the occlusal surface, positioned to reach the mesiobuccal, distobuccal, and palatal canal orifices.

#194 . Endodontic Access Prep

What outline shape is typical for the access cavity on a mandibular molar such as tooth 3.6?

Quick Reference

A rectangular or trapezoidal outline over the mesial two-thirds of the occlusal surface, reaching the mesial and distal canal orifices.

#195 . Endodontic Access Prep

Why does outline form differ by tooth type even though the same four grading criteria apply to every tooth?

Quick Reference

The number and position of root canals differ by tooth type, so the outline shape must be adapted to that root morphology while still achieving optimal straight-line access, complete pulp horn removal, and no overextension beyond a cusp or marginal ridge.

#196 . Endodontic Access Prep

A candidate overextends the access outline beyond a cusp on a molar. Which criterion category grades this, and at what tier?

Quick Reference

External Outline Form, and it is graded Fail; overextension beyond a cusp or marginal ridge is a named Fail condition.

#197 . Endodontic Access Prep

During access preparation the candidate perforates the pulp chamber floor. Which criterion category is affected and what is the outcome?

Quick Reference

Internal Form is graded Fail; perforation is one of the three named Fail conditions for Internal Form on this procedure.

#198 . Endodontic Access Prep

Contrast the canal access depth thresholds for Competent versus Minimally Competent Internal Form.

Quick Reference

Competent requires canals accessed to a full depth of 2.0mm; Minimally Competent allows canals accessed to a depth of less than 2.0mm, provided pulp horns and roof are removed with no perforation.

#199 . Endodontic Access Prep

Under IC&S;, how many total errors are permitted across the entire Day 1 exam, including during the endodontic access procedure?

Quick Reference

A maximum of 3 errors are permitted across the whole day; a 4th Infection Control and Safety error is an automatic fail of the entire Day 1 clinical skills component.

#200 . Endodontic Access Prep

Summarize the single overarching goal that External Outline Form and Internal Form are both grading for on the endodontic access preparation.

Quick Reference

Achieving true straight-line access to every canal, meaning an unobstructed path from the access opening through a fully deroofed chamber with pulp horns removed, down to each canal orifice, without perforation or damaging overextension.

#201 . Crown Preparation

In the NDECC crown preparation criterion, in what jaw position is all clearance checked?

Quick Reference

Clearance is checked in maximum intercuspation, meaning the opposing teeth are fully closed together in their habitual bite when the examiner assesses reduction and clearance.

#202 . Crown Preparation

What is the Competent (optimal) margin position for a crown preparation margin relative to the gingiva?

Quick Reference

The margin is optimally positioned 0.5mm supragingival, meaning it sits half a millimetre coronal to (above) the free gingival margin.

#203 . Crown Preparation

What margin position range is Minimally Competent on the supragingival side?

Quick Reference

A margin positioned supragingival at $\geq 0.0\text{mm}$ and $< 2.0\text{mm}$ is Minimally Competent, a wider but still acceptable band above the gingiva than the 0.5mm optimal target.

#204 . Crown Preparation

At what supragingival margin position does the crown preparation Fail?

Quick Reference

A margin located supragingival at $\geq 2.0\text{mm}$ fails, since a margin that far above the gingiva is considered too high and unacceptable.

#205 . Crown Preparation

What subgingival margin position is Minimally Competent, and what is Fail?

Quick Reference

A margin positioned subgingival at $<1.0\text{mm}$ is Minimally Competent. A margin positioned subgingival at $\geq 1.0\text{mm}$ is a Fail, since burying the margin that deep below the gingiva is unacceptable.

#206 . Crown Preparation

What is the Fail threshold for excessive axial reduction at the margin?

Quick Reference

Excessive axial reduction at the margin greater than 2.0mm is a Fail, since removing that much tooth structure at the margin is considered destructive over-reduction.

#207 . Crown Preparation

What are the two Fail thresholds for insufficient axial reduction at the margin, by surface?

Quick Reference

Insufficient axial reduction is a Fail when the facial surface reduction is less than 0.5mm, or when the lingual surface reduction is less than 0.3mm.

#208 . Crown Preparation

Why does the facial surface have a stricter minimum axial reduction threshold (0.5mm) than the lingual surface (0.3mm) in the NDECC margin criterion?

Quick Reference

The facial minimum is 0.5mm while the lingual minimum is 0.3mm, reflecting that the facial surface generally needs more material clearance for esthetic restorative material thickness than the lingual surface.

#209 . Crown Preparation

What Minimally Competent tolerance exists for axial reduction at the margin measured on the facial surface?

Quick Reference

Axial reduction at the margin within 0.5mm of the required amount on the facial surface is Minimally Competent, allowing a small deviation from the ideal facial reduction before it fails.

#210 . Crown Preparation

What does 'path of draw' mean in a crown preparation?

Quick Reference

Path of draw is the single line of insertion and removal along which a rigid crown restoration can seat onto and be lifted off the prepared tooth without binding on any surface.

#211 . Crown Preparation

What is the Competent standard for path of draw on the NDECC crown prep criterion?

Quick Reference

Competent path of draw shows optimal retention, no undercuts, and allows the restoration to be removed and inserted freely along one consistent path.

#212 . Crown Preparation

What makes a path of draw problem a Fail rather than Minimally Competent?

Quick Reference

It fails when the path of draw actually prevents removal or insertion of the restoration, when excessive convergence compromises retention, or when an undercut requires modification of the prep to correct it.

#213 . Crown Preparation

What undercut condition is Minimally Competent rather than a Fail for path of draw?

Quick Reference

An undercut that can be blocked out with spacer material without any clinical modification of the tooth preparation is Minimally Competent, since it does not require re-cutting the prep.

#214 . Crown Preparation

Define 'undercut' in the context of crown preparation.

Quick Reference

An undercut is any area of the axial wall angled so that it is wider apically (near the gingiva) than occlusally, creating a surface that blocks a rigid crown from seating along a single path of draw.

#215 . Crown Preparation

Define 'taper' (axial convergence) as it applies to crown preparation walls.

Quick Reference

Taper, or axial convergence, is the inward angle of the opposing axial walls from gingival to occlusal; a small taper toward the occlusal creates retention while walls that are too parallel or too convergent affect fit and retention.

#216 . Crown Preparation

What is Minimally Competent regarding axial wall taper in the path of draw criterion?

Quick Reference

Excessive axial convergence that does not compromise retention is Minimally Competent, as is a set of parallel axial walls that still allows a provisional restoration to be seated and removed.

#217 . Crown Preparation

Define 'axial reduction' in crown preparation.

Quick Reference

Axial reduction is the removal of tooth structure from the vertical (axial) walls of the tooth, done in the amount specified for the restoration material to create room for it while preserving retention form.

#218 . Crown Preparation

Define 'occlusal reduction' in crown preparation.

Quick Reference

Occlusal reduction is the removal of tooth structure from the biting (occlusal or incisal) surface to create adequate thickness of space for the restorative material without interfering with the opposing dentition.

#219 . Crown Preparation

What criterion covers occlusal or incisal reduction amount on the NDECC crown prep grading table?

Quick Reference

It falls under Preservation of Tooth Vitality and Structural Durability, where Competent is optimal reduction for the specified restoration and Fail includes insufficient reduction preventing fabrication or reduction negatively impacting the occlusal plane.

#220 . Crown Preparation

What is a Fail for under-reduction under Preservation of Tooth Vitality and Structural Durability?

Quick Reference

Insufficient reduction that prevents fabrication of the restoration is a Fail, as is any incisal or occlusal reduction that negatively impacts the occlusal plane.

#221 . Crown Preparation

What is a Fail for over-reduction under Preservation of Tooth Vitality and Structural Durability?

Quick Reference

Reduction that compromises the vitality of the tooth or the structural integrity of the remaining tooth is a Fail, since removing too much dentin risks pulp exposure or a weakened tooth.

#222 . Crown Preparation

What is Minimally Competent for over-reduction and under-reduction on this criterion?

Quick Reference

Over-reduction that does not impact vitality or structural integrity is Minimally Competent, and under-reduction that does not impact occlusion, contour, or structural integrity is also Minimally Competent.

#223 . Crown Preparation

Define 'finish line' in crown preparation and how it relates to 'margin'.

Quick Reference

The finish line is the terminal border of the tooth preparation where the prepared tooth surface meets the unprepared tooth or the future restoration edge; it is the same line referred to as the margin when discussing the crown's fit and position.

#224 . Crown Preparation

Define 'margin design' as assessed in crown preparation.

Quick Reference

Margin design is the cross-sectional shape cut at the finish line (for example chamfer, shoulder, or knife-edge) chosen to match the restorative material's strength and esthetic requirements at that location.

#225 . Crown Preparation

What happens on the Margin criterion if the wrong margin design is cut for the specified restoration?

Quick Reference

An incorrect margin design is a Fail, since a mismatched finish-line shape cannot properly support the specified restorative material at the margin.

#226 . Crown Preparation

What margin continuity issue is a Fail versus Minimally Competent?

Quick Reference

A discontinuous or nondiscernible margin is a Fail, while a margin that is continuous but has a minor irregularity is Minimally Competent.

#227 . Crown Preparation

What is the Competent standard on the Margin criterion overall?

Quick Reference

Competent means the margin is optimally defined, positioned 0.5mm supragingival, has axial reduction matching the specified restoration, and uses the correct margin design.

#228 . Crown Preparation

What are the categories graded on the NDECC crown preparation procedure?

Quick Reference

Crown preparation is graded on Path of Draw, Preservation of Tooth Vitality and Structural Durability, Margin, Finish, and Damage.

#229 . Crown Preparation

What is the Competent standard for the Finish criterion in crown preparation?

Quick Reference

Competent finish means smooth prepared surfaces overall, with no sharp line angles or cusps and no unsupported enamel.

#230 . Crown Preparation

What sharp line angle or cusp finding is Minimally Competent versus Fail?

Quick Reference

A sharp line angle or cusp that does not compromise structural integrity is Minimally Competent; a sharp line angle or cusp that does compromise integrity is a Fail.

#231 . Crown Preparation

What roughness finding is Minimally Competent on the Finish criterion?

Quick Reference

Roughness that does not impact fabrication of the restoration or its retention is Minimally Competent, as is unsupported enamel that does not compromise marginal integrity.

#232 . Crown Preparation

What Finish findings are automatic Fails?

Quick Reference

A sharp line angle or cusp that compromises structural integrity, unsupported enamel that compromises marginal integrity, and tooth structure rebuilt with restorative material are all Fails on the Finish criterion.

#233 . Crown Preparation

What is the Damage criterion pattern for crown preparation, and how does it compare across procedures?

Quick Reference

Damage follows the same pattern used across all Day 1 procedures: damage correctable by enameloplasty is Minimally Competent, while damage requiring a restoration is a Fail.

#234 . Crown Preparation

What are the Critical Errors for crown preparation, and what do Critical Errors mean overall?

Quick Reference

The Critical Errors for crown preparation are anything that prevents evaluation of the exam tooth, or no preparation performed at all; any Critical Error is an automatic Fail of the procedure regardless of other criteria scores.

#235 . Crown Preparation

Why is the same tooth prep used for the Provisional Crown Restoration procedure on the NDECC?

Quick Reference

The provisional crown restoration procedure is fabricated on the candidate's own crown preparation, so a poor path of draw, wrong margin position, or missing retention form on the prep can directly cause the provisional to fit, seat, or retain poorly.

#236 . Crown Preparation

A crown margin is measured at 1.5mm supragingival. Which tier does this fall into?

Quick Reference

Minimally Competent, since supragingival margin position from $\geq 0.0\text{mm}$ to $< 2.0\text{mm}$ is Minimally Competent; only $\geq 2.0\text{mm}$ supragingival is a Fail.

#237 . Crown Preparation

A crown margin is measured at 1.2mm subgingival. Which tier does this fall into?

Quick Reference

Fail, since subgingival margin position $\geq 1.0\text{mm}$ is a Fail; only subgingival positions less than 1.0mm are Minimally Competent.

#238 . Crown Preparation

Facial axial reduction at the margin measures 0.4mm. Which tier does this fall into?

Quick Reference

Fail, since insufficient axial reduction at the margin is defined as facial reduction less than 0.5mm.

#239 . Crown Preparation

Lingual axial reduction at the margin measures 0.4mm. Which tier does this fall into?

Quick Reference

This is acceptable (not a Fail) on the lingual surface, since the lingual insufficient-reduction Fail threshold is less than 0.3mm and 0.4mm exceeds that minimum.

#240 . Crown Preparation

Axial reduction at the margin measures 2.3mm. Which tier does this fall into?

Quick Reference

Fail, since excessive axial reduction at the margin is defined as greater than 2.0mm.

#241 . Crown Preparation

What criterion assesses whether a restoration can be seated onto and removed from the prepared tooth without binding?

Quick Reference

Path of Draw assesses this, checking that the axial walls create one consistent line of insertion and removal with no blocking undercuts.

#242 . Crown Preparation

How does 'excessive convergence' differ in severity from 'parallel axial walls' under the Path of Draw criterion?

Quick Reference

Excessive convergence that does not compromise retention is Minimally Competent, and truly parallel axial walls that still allow a provisional to be drawn on and off are also Minimally Competent; only convergence severe enough to compromise retention, or an undercut requiring modification, is a Fail.

#243 . Crown Preparation

What is the correct FDI tooth notation style used throughout the NDECC crown preparation content, and why does it matter?

Quick Reference

FDI two-digit notation such as tooth 4.6 or tooth 1.1 is used throughout, not the American Universal Numbering System (1 to 32), since NDECC is a Canadian exam and grading materials use FDI notation.

#244 . Crown Preparation

Besides Path of Draw and Margin, name the remaining criteria categories for crown preparation and one Fail example from each.

Quick Reference

Preservation of Tooth Vitality and Structural Durability fails on insufficient reduction preventing fabrication; Finish fails on unsupported enamel compromising marginal integrity; Damage fails when damage requires a restoration.

#245 . Crown Preparation

Summarize the four numeric axial reduction and margin position thresholds that most often decide a crown preparation Fail.

Quick Reference

Facial axial reduction below 0.5mm, lingual axial reduction below 0.3mm, axial reduction at the margin above 2.0mm, and margin position at or beyond 2.0mm supragingival or at or beyond 1.0mm subgingival are the four thresholds that most often decide a Fail.

#246 . Provisional Crown Restoration

Which of the 7 Day 1 Clinical Skills procedures is the provisional crown restoration, and what is it fabricated on?

Quick Reference

It is procedure 7 of 7. It is fabricated on the candidate's own crown preparation from procedure 6, so a poor crown prep can make the provisional harder to fit correctly.

#247 . Provisional Crown Restoration

How many grading categories does the provisional crown restoration use, and what are they?

Quick Reference

Three categories: Margin and Adaptation, Morphology and Occlusion, and Finish.

#248 . Provisional Crown Restoration

Does the provisional crown restoration have a Damage category like most other Day 1 procedures?

Quick Reference

No. Provisional crown restoration is graded on only three categories (Margin and Adaptation, Morphology and Occlusion, Finish) with no separate Damage category.

#249 . Provisional Crown Restoration

What defines Competent on the Margin and Adaptation criterion for a provisional crown?

Quick Reference

Optimal margin, optimal adaptation, optimal retention, optimal stability, optimal seating, and the restoration is removable and intact.

#250 . Provisional Crown Restoration

An underextended margin that does not impact retention is graded how on Margin and Adaptation?

Quick Reference

Minimally Competent.

#251 . Provisional Crown Restoration

An undercontoured margin that does not impact durability is graded how on Margin and Adaptation?

Quick Reference

Minimally Competent.

#252 . Provisional Crown Restoration

An overextended or overcontoured margin that does not impact gingival health is graded how on Margin and Adaptation?

Quick Reference

Minimally Competent.

#253 . Provisional Crown Restoration

For Margin and Adaptation, what two additional findings are acceptable at the Minimally Competent tier (beyond margin issues)?

Quick Reference

Fully seated, stable, and retentive; and removable and intact.

#254 . Provisional Crown Restoration

An underextended margin negatively impacting retention or causing sensitivity is graded how on Margin and Adaptation?

Quick Reference

Fail.

#255 . Provisional Crown Restoration

An overcontoured margin negatively impacting gingival health is graded how on Margin and Adaptation?

Quick Reference

Fail.

#256 . Provisional Crown Restoration

An overextended or undercontoured margin resulting in poor structural durability is graded how on Margin and Adaptation?

Quick Reference

Fail.

#257 . Provisional Crown Restoration

What are the two stability-related Fail findings on the Margin and Adaptation criterion?

Quick Reference

Restoration is unstable; and restoration cannot be removed.

#258 . Provisional Crown Restoration

Why must a provisional crown be removable, and what happens if it cannot be removed?

Quick Reference

The provisional is a temporary restoration meant to protect the prep between visits, so it must come off intact when required. A provisional that cannot be removed is a Fail on Margin and Adaptation.

#259 . Provisional Crown Restoration

What defines Competent on the Morphology and Occlusion criterion for a provisional crown?

Quick Reference

Optimal contour for gingival health and esthetics, plus optimal interproximal and occlusal contacts.

#260 . Provisional Crown Restoration

An over or undercontour that does not affect occlusal contacts is graded how on Morphology and Occlusion?

Quick Reference

Minimally Competent.

#261 . Provisional Crown Restoration

A proximal contact that is present but small, light, or not optimal is graded how on Morphology and Occlusion?

Quick Reference

Minimally Competent.

#262 . Provisional Crown Restoration

Supraocclusion with light contacts elsewhere in the quadrant is graded how on Morphology and Occlusion?

Quick Reference

Minimally Competent.

#263 . Provisional Crown Restoration

A restoration that does not reproduce anatomical contour, negatively impacting gingival health, esthetics, or durability, is graded how on Morphology and Occlusion?

Quick Reference

Fail.

#264 . Provisional Crown Restoration

What two proximal contact findings are Fail on Morphology and Occlusion?

Quick Reference

Absent proximal contact (contact is not present), or a contact that blocks floss entirely.

#265 . Provisional Crown Restoration

An occlusal contact risking fracture or trauma is graded how on Morphology and Occlusion?

Quick Reference

Fail.

#266 . Provisional Crown Restoration

A provisional crown that fractures or cracks during occlusion evaluation is graded how on Morphology and Occlusion?

Quick Reference

Fail. This is distinct from submitting a restoration that is already fractured, which is a Critical Error.

#267 . Provisional Crown Restoration

What defines Competent on the Finish criterion for a provisional crown?

Quick Reference

Smooth, optimally polished, no roughness or porosities, and no excess material on tissue.

#268 . Provisional Crown Restoration

Roughness that allows floss through without breaking it is graded how on Finish?

Quick Reference

Minimally Competent.

#269 . Provisional Crown Restoration

Roughness or porosities unlikely to cause discomfort or food retention are graded how on Finish?

Quick Reference

Minimally Competent.

#270 . Provisional Crown Restoration

Roughness that causes floss to break is graded how on Finish for a provisional crown?

Quick Reference

Fail.

#271 . Provisional Crown Restoration

Roughness or porosity causing discomfort or food retention is graded how on Finish?

Quick Reference

Fail.

#272 . Provisional Crown Restoration

Excess restorative material left on soft or hard tissue is graded how on Finish?

Quick Reference

Fail.

#273 . Provisional Crown Restoration

What are the four Critical Errors for provisional crown restoration?

Quick Reference

Anything preventing evaluation; no restoration performed; restoration submitted fractured; and foreign objects or materials remaining in the oral cavity.

#274 . Provisional Crown Restoration

If a candidate submits a provisional crown that is already fractured, is this a Fail or a Critical Error?

Quick Reference

A Critical Error, which is an automatic fail regardless of any other criteria scored. This is different from a crown that fractures during the exam's own occlusion evaluation, which is scored as a Fail under Morphology and Occlusion.

#275 . Provisional Crown Restoration

Leaving no restoration at all on the crown prep is graded how on the provisional crown procedure?

Quick Reference

Critical Error, an automatic fail.

#276 . Provisional Crown Restoration

What is a pre-formed shell in provisional crown fabrication?

Quick Reference

A ready-made temporary crown shell (for example polycarbonate or metal) selected and adapted to the candidate's own crown prep, rather than shaped entirely by hand from raw material.

#277 . Provisional Crown Restoration

What is free-hand fabrication of a provisional crown?

Quick Reference

Building the provisional directly on or from an impression of the prepared tooth using a moldable material, shaping the crown entirely by hand rather than starting from a pre-formed shell.

#278 . Provisional Crown Restoration

What is trimming in the context of provisional crown fabrication?

Quick Reference

Removing excess provisional material at the margins and contours so the restoration seats fully, does not overextend or overcontour the margin, and does not leave excess material on tissue.

#279 . Provisional Crown Restoration

What is polishing in the context of provisional crown fabrication?

Quick Reference

Smoothing the finished provisional's surfaces to eliminate roughness and porosities, supporting a Competent Finish grade and reducing the risk of floss breaking or food and plaque retention.

#280 . Provisional Crown Restoration

Why does trimming matter directly for the Margin and Adaptation criterion?

Quick Reference

Overextended or overcontoured margins from insufficient trimming can be graded Minimally Competent if gingival health is not impacted, or Fail if gingival health is negatively impacted or structural durability suffers.

#281 . Provisional Crown Restoration

Why does polishing matter directly for the Finish criterion?

Quick Reference

Because Competent Finish requires the provisional be smooth and optimally polished with no roughness or porosities; incomplete polishing that lets floss break or causes discomfort or food retention drops the grade to Fail.

#282 . Provisional Crown Restoration

Name the three tiers used to grade every criterion on the provisional crown restoration.

Quick Reference

Competent (pass, optimal), Minimally Competent (pass, acceptable), and Not Competent, also called Fail.

#283 . Provisional Crown Restoration

Is there partial credit on the provisional crown restoration procedure?

Quick Reference

No. Like all 7 Day 1 Clinical Skills procedures, it is graded Pass or Fail overall with no partial credit or percentage score, though each of the three criteria categories is assessed at its own tier.

#284 . Provisional Crown Restoration

How does the Margin and Adaptation criterion's removable requirement differ from the Critical Errors list?

Quick Reference

A restoration that is not fully removable when it should be is scored as a Fail under Margin and Adaptation, while submitting a restoration that is fractured is a listed Critical Error, an automatic fail regardless of other scores.

#285 . Provisional Crown Restoration

Summarize the provisional crown restoration's grading structure in one sentence.

Quick Reference

Three criteria (Margin and Adaptation, Morphology and Occlusion, Finish), each tiered Competent/Minimally Competent/Fail with no Damage category, plus four Critical Errors (evaluation prevented, no restoration performed, restoration submitted fractured, foreign objects remaining) that are automatic fails.

#286 . SJ: Patient-Centered Care

What is the format of Day 2 of the NDECC?

Quick Reference

A half-day (about 4 hours) Situational Judgement component with 10 stations, 2 stations in each of 5 domains: Patient-Centered Care, Professionalism, Communication and Collaboration, Practice and Information Management, and Health Promotion.

#287 . SJ: Patient-Centered Care

How is each Situational Judgement station graded?

Quick Reference

Pass/Fail, using a weighted-behavior checklist (rubric), not a percentage score.

#288 . SJ: Patient-Centered Care

What is the overall pass rule for the Situational Judgement component?

Quick Reference

The candidate must pass at least 6 of the 10 stations, AND at least 1 of the 2 stations in EACH of the 5 domains. Failing both stations in one domain is an automatic overall fail even if 6 stations overall were passed.

#289 . SJ: Patient-Centered Care

In a Situational Judgement rubric, how many points are Competent behaviors worth?

Quick Reference

Zero points. Competent behaviors are nice-to-have actions that do not by themselves affect the pass or fail outcome of the station.

#290 . SJ: Patient-Centered Care

In a Situational Judgement rubric, what must the Minimally Competent behavior points sum to in order to pass a station?

Quick Reference

Exactly 1.0. Each Minimally Competent behavior carries a point value (for example 0.20 each), and the candidate must accumulate a total of 1.0 across the behaviors actually demonstrated.

#291 . SJ: Patient-Centered Care

What happens if a candidate commits a listed Critical Error during a Situational Judgement station?

Quick Reference

It is an automatic fail (worth -1) for that station, regardless of how many Minimally Competent points were earned.

#292 . SJ: Patient-Centered Care

In the worked tooth 4.1 example, what is the patient's presenting complaint?

Quick Reference

Low-grade pain on tooth 4.1, a tooth restored years earlier, with a periapical radiograph showing probing depths of 2 to 3mm; the candidate must discuss treatment options with the patient.

#293 . SJ: Patient-Centered Care

In the worked tooth 4.1 example, name the four Competent (0-point) behaviors.

Quick Reference

Answers any questions; gives the patient an opportunity to ask questions; discusses the patient's special circumstances if applicable; discusses fees.

#294 . SJ: Patient-Centered Care

In the worked tooth 4.1 example, what point value is assigned to each Minimally Competent behavior?

Quick Reference

0.20 points each, so all five listed Minimally Competent behaviors sum to exactly 1.0 if all are demonstrated.

#295 . SJ: Patient-Centered Care

In the worked tooth 4.1 example, list the five Minimally Competent behaviors.

Quick Reference

Summarizes the problems with the tooth; provides more than one complete treatment option; explains the advantages and disadvantages of the different options and the preferred option; verifies the patient understands the explanations; communicates information effectively and in an organized manner.

#296 . SJ: Patient-Centered Care

Does the tooth 4.1 example require the candidate to recommend only one treatment option?

Quick Reference

No. One point-bearing behavior specifically requires providing more than one complete treatment option, not a single option.

#297 . SJ: Patient-Centered Care

Why must a candidate verify the patient understands the explanations in an SJ station like the tooth 4.1 example?

Quick Reference

Because verifying understanding is one of the point-bearing Minimally Competent behaviors; simply giving information without checking comprehension does not earn that point.

#298 . SJ: Patient-Centered Care

What are the five official example sub-skills of the Patient-Centered Care domain?

Quick Reference

Manage patients' needs; develop a problem list; establish diagnoses; sequence treatment; develop a treatment plan.

#299 . SJ: Patient-Centered Care

What does managing patients' needs mean as a Patient-Centered Care sub-skill?

Quick Reference

Recognizing and responding to what the patient actually needs, such as pain relief, information, reassurance, or referral, not only the clinical finding itself.

#300 . SJ: Patient-Centered Care

What does developing a problem list mean as a Patient-Centered Care sub-skill?

Quick Reference

Identifying and organizing every clinical issue affecting the patient (for example pain, a defective restoration, periodontal findings) before deciding on treatment.

#301 . SJ: Patient-Centered Care

What does establishing diagnoses mean as a Patient-Centered Care sub-skill?

Quick Reference

Turning the findings on the problem list into specific diagnoses, such as identifying the cause of tooth 4.1's pain, rather than only describing symptoms.

#302 . SJ: Patient-Centered Care

What does sequencing treatment mean as a Patient-Centered Care sub-skill?

Quick Reference

Placing the needed treatments in the correct clinical order, for example addressing an acute or urgent problem before elective or cosmetic care.

#303 . SJ: Patient-Centered Care

What does developing a treatment plan mean as a Patient-Centered Care sub-skill?

Quick Reference

Assembling the diagnoses and sequence into a complete plan of care that is presented to and discussed with the patient, including options and the preferred option.

#304 . SJ: Patient-Centered Care

In a Patient-Centered Care SJ station, is discussing fees a point-bearing behavior?

Quick Reference

No. In the tooth 4.1 example, discussing fees is one of the 0-point Competent behaviors, not one of the point-bearing Minimally Competent behaviors.

#305 . SJ: Patient-Centered Care

What is the risk of presenting only one treatment option in a Patient-Centered Care station?

Quick Reference

The candidate misses the point-bearing behavior of providing more than one complete treatment option, reducing the chance of reaching the 1.0 points needed to pass.

#306 . SJ: Patient-Centered Care

How many domains make up the Situational Judgement component and how many stations are in each?

Quick Reference

5 domains, each with 2 stations, for a total of 10 stations.

#307 . SJ: Patient-Centered Care

Name all 5 Situational Judgement domains.

Quick Reference

Patient-Centered Care, Professionalism, Communication and Collaboration, Practice and Information Management, Health Promotion.

#308 . SJ: Patient-Centered Care

If a candidate passes 7 of 10 SJ stations overall but fails both Patient-Centered Care stations, do they pass Day 2?

Quick Reference

No. Even though 6 of 10 was reached, failing both stations in one domain (Patient-Centered Care) is an automatic overall fail for the domain requirement.

#309 . SJ: Patient-Centered Care

If a candidate passes exactly 1 station in each of the 5 domains (5 of 10 total), do they pass Day 2?

Quick Reference

No. Passing 1 of 2 per domain satisfies the per-domain rule but only reaches 5 of 10 stations overall, short of the required 6 of 10.

#310 . SJ: Patient-Centered Care

What two conditions must BOTH be satisfied to pass the Situational Judgement component?

Quick Reference

At least 6 of the 10 stations passed overall, AND at least 1 of the 2 stations passed in every one of the 5 domains.

#311 . SJ: Patient-Centered Care

Is the Situational Judgement rubric shape the same across all 10 stations?

Quick Reference

Yes. Every station uses the same template: 0-point Competent behaviors, point-bearing Minimally Competent behaviors summing to 1.0, and a list of station-specific Critical Errors that cause automatic fail.

#312 . SJ: Patient-Centered Care

Can a station's list of Minimally Competent behaviors have a different number of items from station to station?

Quick Reference

Yes. The number of Minimally Competent behaviors can vary by scenario, but whatever the count, their point values must always sum to exactly 1.0.

#313 . SJ: Patient-Centered Care

Are Critical Errors the same list for every Situational Judgement station?

Quick Reference

No. Critical Errors are specific to each station's scenario, but any listed Critical Error always results in an automatic fail for that station regardless of domain.

#314 . SJ: Patient-Centered Care

In the tooth 4.1 scenario, what clinical finding is provided on the periapical radiograph?

Quick Reference

Probing depths of 2 to 3mm around tooth 4.1.

#315 . SJ: Patient-Centered Care

In a Patient-Centered Care station, what counts as developing a problem list for a patient like the one in the tooth 4.1 scenario?

Quick Reference

Recording all relevant findings, such as the low-grade pain, the existing old restoration, and the periodontal probing depths, as distinct problems before moving to diagnosis.

#316 . SJ: Patient-Centered Care

Why is giving the patient an opportunity to ask questions only a Competent (0-point) behavior rather than point-bearing?

Quick Reference

Because it is considered a baseline good-practice behavior expected of every competent candidate, not a distinguishing behavior that separates minimally competent performance from failing performance.

#317 . SJ: Patient-Centered Care

What is the practical implication of Competent behaviors being worth 0 points?

Quick Reference

Demonstrating only Competent behaviors, without any Minimally Competent behaviors, will not accumulate the 1.0 points needed to pass the station.

#318 . SJ: Patient-Centered Care

How should a candidate approach sequencing treatment when a patient has both an urgent problem and a long-term restorative need?

Quick Reference

Address the urgent or acute problem first, then plan the longer-term restorative or elective treatment afterward, reflecting correct clinical priority.

#319 . SJ: Patient-Centered Care

What is the difference between a problem list and a treatment plan in the Patient-Centered Care domain?

Quick Reference

A problem list identifies and organizes the clinical issues found; a treatment plan is the resulting sequenced set of interventions, including options, chosen to address those problems.

#320 . SJ: Patient-Centered Care

In the tooth 4.1 example, what does communicates information effectively and in an organized manner assess?

Quick Reference

Whether the candidate presents the summary of problems, treatment options, and recommendation in a clear, logical, easy-to-follow way, not just whether the correct facts were mentioned.

#321 . SJ: Patient-Centered Care

Why does the tooth 4.1 rubric include discusses the patient's special circumstances if applicable as only a 0-point behavior?

Quick Reference

It is conditional (only applies if special circumstances exist) and is treated as a baseline good-practice consideration rather than a behavior that determines minimal competence.

#322 . SJ: Patient-Centered Care

What is one way a candidate could earn 0.60 of the 1.0 required points in the tooth 4.1 station?

Quick Reference

By demonstrating any three of the five 0.20-point Minimally Competent behaviors, for example summarizing the problems, giving more than one treatment option, and communicating effectively, while still needing 0.40 more points from the remaining behaviors to pass.

#323 . SJ: Patient-Centered Care

Does earning all four Competent behaviors without any Minimally Competent behaviors pass the tooth 4.1 station?

Quick Reference

No. Since Competent behaviors are worth 0 points, the candidate would have 0 of the 1.0 points required, resulting in a fail.

#324 . SJ: Patient-Centered Care

What is the purpose of establishing diagnoses before developing a treatment plan in the Patient-Centered Care workflow?

Quick Reference

A treatment plan should be built on confirmed diagnoses so that each planned intervention directly addresses an identified cause, rather than only treating symptoms.

#325 . SJ: Patient-Centered Care

In the Patient-Centered Care domain, what does managing patients' needs add beyond simply making a correct clinical diagnosis?

Quick Reference

It requires attending to what matters to the patient, such as pain control, anxiety, understanding, and practical concerns, alongside the technical diagnosis and plan.

#326 . SJ: Patient-Centered Care

How many total Situational Judgement stations must a candidate pass at minimum to meet the overall 6-of-10 rule?

Quick Reference

6 stations out of the 10 total, in addition to satisfying the separate 1-of-2-per-domain rule.

#327 . SJ: Patient-Centered Care

Is a percentage score used to determine pass or fail on an individual Situational Judgement station?

Quick Reference

No. Each station is graded strictly Pass/Fail based on whether Minimally Competent points sum to 1.0 and no Critical Error occurred, not by a percentage.

#328 . SJ: Patient-Centered Care

What administrative detail is NOT officially published about the Situational Judgement stations?

Quick Reference

The exact per-station timing in minutes; NDEB frames Day 2 as roughly a half day of about 4 hours across 10 stations, but does not publish the exact minutes per station.

#329 . SJ: Patient-Centered Care

In a Patient-Centered Care station, what is a practical way to remember the 5 sub-skills in workflow order?

Quick Reference

Manage the patient's needs, develop a problem list, establish diagnoses, sequence the treatment, then develop the treatment plan, moving from listening to the patient through to a final organized plan.

#330 . SJ: Patient-Centered Care

Why does presenting the preferred option along with advantages and disadvantages of the different options matter for scoring?

Quick Reference

It is a single combined point-bearing Minimally Competent behavior in the tooth 4.1 example: explaining the advantages and disadvantages of different options AND the preferred option, so omitting the preferred-option recommendation could cost that 0.20 point.

#331 . SJ: Professionalism

What is Day 2 of the NDECC called and how many stations does it have?

Quick Reference

The Situational Judgement component, a half day of about 4 hours with 10 stations, 2 stations in each of 5 domains.

#332 . SJ: Professionalism

What is the overall pass rule across all 10 Situational Judgement stations?

Quick Reference

You must pass at least 6 of the 10 stations, and pass at least 1 of the 2 stations in every single domain. Failing both stations in one domain fails the exam regardless of your total score.

#333 . SJ: Professionalism

What are the three tiers of behavior in a Situational Judgement rubric?

Quick Reference

Competent behaviors (0 points, nice to have), Minimally Competent behaviors (point-bearing, must sum to 1.0 to pass), and Critical Errors (automatic fail regardless of points earned).

#334 . SJ: Professionalism

How many points must the Minimally Competent behaviors sum to for a station to pass?

Quick Reference

Exactly 1.0. Each Minimally Competent behavior typically carries a value such as 0.20, and enough of them must be demonstrated to reach the 1.0 total.

#335 . SJ: Professionalism

Does earning a high point total protect you from a Critical Error in a Situational Judgement station?

Quick Reference

No. Any listed Critical Error is an automatic fail for that station regardless of how many Minimally Competent points were earned.

#336 . SJ: Professionalism

Name the 5 domains tested on Day 2 of the NDECC.

Quick Reference

Patient-Centered Care, Professionalism, Communication and Collaboration, Practice and Information Management, and Health Promotion.

#337 . SJ: Professionalism

According to NDEB's own protocol, what are the three example sub-skills under the Professionalism domain?

Quick Reference

Responding to ethical dilemmas, managing referrals, and using social media appropriately.

#338 . SJ: Professionalism

A colleague's restoration you are asked to redo is clearly defective. What professionalism principle guides how you discuss this with the patient?

Quick Reference

Honest, non-judgemental disclosure focused on the patient's current clinical needs, avoiding disparaging remarks about the previous dentist while still giving the patient accurate information to make decisions.

#339 . SJ: Professionalism

You notice a colleague made a clinical error that harmed a patient. What is the professionally correct response?

Quick Reference

Disclose the error and its consequences to the affected patient honestly and promptly, encourage or support the treating dentist in disclosing it, and document objectively without assigning blame in the patient record.

#340 . SJ: Professionalism

A patient asks you to refer them to a specialist, but you would prefer to keep the case yourself for the fee. What is the ethical obligation?

Quick Reference

Referral decisions must be based on the patient's best interest and your own scope of competence, not on financial gain. A conflict of interest must never override the duty of care owed to the patient.

#341 . SJ: Professionalism

What is a conflict of interest in dental practice?

Quick Reference

A situation where a dentist's personal, financial, or other interests could improperly influence professional judgement or the care recommended to a patient.

#342 . SJ: Professionalism

How should a dentist properly manage a genuine conflict of interest?

Quick Reference

Disclose the conflict transparently to the patient, prioritize the patient's welfare over personal gain, and if needed step aside or refer the patient to another practitioner.

#343 . SJ: Professionalism

What is the correct process when managing a specialist referral for a patient?

Quick Reference

Select the referral based on the patient's clinical needs and the specialist's competence, prepare complete and accurate referral documentation, communicate the reason for referral clearly to both the patient and the specialist, and follow up on outcomes.

#344 . SJ: Professionalism

A patient's third molar case is beyond your scope of competence. What professionalism sub-skill applies?

Quick Reference

Managing referrals: recognizing your own limits and referring the patient to an appropriately qualified colleague or specialist rather than treating beyond your competence.

#345 . SJ: Professionalism

Is it acceptable to post identifiable before-and-after photos of a patient's treatment on social media without consent?

Quick Reference

No. Using social media appropriately requires explicit informed patient consent before posting any identifiable clinical images, and confidentiality must be protected at all times.

#346 . SJ: Professionalism

A patient leaves a negative online review naming specific treatment details. Can you respond publicly with the patient's clinical information to defend yourself?

Quick Reference

No. Disclosing a patient's confidential clinical information in a public response, even to correct a review, is a confidentiality breach. Respond generally and invite the patient to discuss privately instead.

#347 . SJ: Professionalism

What is the core professionalism principle behind using social media appropriately?

Quick Reference

Maintaining patient confidentiality, obtaining consent for any patient-related content, avoiding misleading or unsubstantiated claims, and upholding the same standards of conduct online as in the clinic.

#348 . SJ: Professionalism

A dentist wants to post a testimonial-style photo of a patient's smile makeover. What must happen first?

Quick Reference

The patient must give explicit, informed, and preferably written consent specifically for that use, understanding the image may be identifiable and publicly shared.

#349 . SJ: Professionalism

What defines a professional boundary violation with a patient?

Quick Reference

Any personal, financial, or social relationship with a patient that compromises objective clinical judgement or exploits the trust and power imbalance inherent in the dentist-patient relationship.

#350 . SJ: Professionalism

Is it appropriate to begin a romantic relationship with a current patient?

Quick Reference

No. This is a professional boundary violation because the therapeutic relationship creates a power imbalance that can compromise objectivity and patient welfare.

#351 . SJ: Professionalism

A close friend asks you to treat them and waive standard consent and record-keeping steps because you know each other well. How should you respond?

Quick Reference

Maintain the same informed consent, documentation, and treatment standards used for any patient. Personal relationships do not exempt a dentist from professional obligations.

#352 . SJ: Professionalism

What is an ethical dilemma, in the context of the Professionalism domain?

Quick Reference

A situation where two or more professional or ethical duties conflict, such as patient autonomy versus beneficence, or confidentiality versus a duty to warn, and there is no single obviously correct action.

#353 . SJ: Professionalism

A capable adult patient refuses a recommended treatment you believe is clearly in their best interest. What principle should guide your response?

Quick Reference

Respect patient autonomy. Provide full information about risks and benefits, verify the patient understands, document the discussion, and accept their informed decision without coercion.

#354 . SJ: Professionalism

A patient discloses they are being abused at home during a routine appointment. What is the professional obligation?

Quick Reference

Follow applicable provincial mandatory reporting requirements where relevant, prioritize patient safety, respond with sensitivity, and document the disclosure and actions taken.

#355 . SJ: Professionalism

A minor patient discloses information they ask you to keep secret from their parent. What tension does this create?

Quick Reference

A tension between confidentiality and the parent or guardian's legal right to information about a minor's care; the response depends on the minor's age, maturity, and provincial consent-to-treatment law.

#356 . SJ: Professionalism

A patient's insurance company asks for clinical details beyond what is needed to process a claim. What should you do?

Quick Reference

Disclose only the minimum information necessary for the specific claim and obtain patient consent before releasing any additional clinical information.

#357 . SJ: Professionalism

What is the professional response if you discover you made a clinical error yourself?

Quick Reference

Disclose the error to the patient honestly and promptly, explain the implications and corrective options, document the incident objectively, and take steps to remedy the harm.

#358 . SJ: Professionalism

A colleague asks you to co-sign a chart entry for treatment they performed but you did not witness. What is the correct response?

Quick Reference

Decline. Signing or co-signing documentation for care you did not personally provide or witness is dishonest and compromises the accuracy of the patient record.

#359 . SJ: Professionalism

You suspect a colleague is practising while impaired by substance use. What is the professional obligation?

Quick Reference

Prioritize patient safety, and report the concern through the appropriate regulatory or professional channel, since failing to act could allow patient harm to continue.

#360 . SJ: Professionalism

A dental supply representative offers you a personal gift tied to prescribing a specific product. What ethical concern does this raise?

Quick Reference

A conflict of interest: accepting the gift could improperly influence your clinical product choices away from what is genuinely best for the patient.

#361 . SJ: Professionalism

What should referral documentation always include, per the Professionalism domain?

Quick Reference

The clinical reason for referral, relevant history and findings, radiographs or records as needed, and clear communication of urgency, so the receiving practitioner has what is needed to continue care safely.

#362 . SJ: Professionalism

A patient asks you to falsify a treatment date on an insurance form so an expired benefit will still be covered. What is the correct response?

Quick Reference

Refuse. Falsifying records or claims is a serious ethical and legal violation, regardless of the patient's financial hardship or intent.

#363 . SJ: Professionalism

What professional value is tested when a Situational Judgement station involves accepting a gift from a patient?

Quick Reference

Maintaining appropriate boundaries and objectivity; a small token gift is usually acceptable, but a gift substantial enough to influence clinical judgement or create obligation should be respectfully declined or disclosed.

#364 . SJ: Professionalism

In the worked Patient-Centered Care example from NDEB's protocol, what value did EACH Minimally Competent behavior carry, and how many were listed?

Quick Reference

Each Minimally Competent behavior carried 0.20 points, and 5 were listed, summing to exactly 1.0 to pass.

#365 . SJ: Professionalism

True or false: a Situational Judgement station's Competent behaviors are required to pass the station.

Quick Reference

False. Competent behaviors are worth 0 points and are considered nice to have; only the point-bearing Minimally Competent behaviors summing to 1.0 determine a pass, alongside the absence of any Critical Error.

#366 . SJ: Professionalism

A patient asks for your personal opinion on a competing dentist's treatment quality on a public review site. How should you respond?

Quick Reference

Decline to comment publicly on a colleague's care. Offer to discuss the patient's own clinical situation privately instead, protecting both confidentiality and professional courtesy.

#367 . SJ: Professionalism

What is the risk of accepting a patient's Facebook friend request or following them on personal social media?

Quick Reference

It can blur professional boundaries, expose private information inappropriately, and create dual relationships that compromise objective clinical judgement.

#368 . SJ: Professionalism

A patient with a genuine emergency requests urgent transfer of their records to a new dentist. Can you delay release pending an outstanding balance?

Quick Reference

No. Withholding records needed for urgent or ongoing care over an unpaid balance can jeopardize patient welfare and is a professionalism and ethical violation; timely record transfer takes priority.

#369 . SJ: Professionalism

What is the underlying theme connecting all three Professionalism sub-skills: ethical dilemmas, referrals, and social media?

Quick Reference

Each requires putting the patient's welfare, informed consent, and confidentiality ahead of the dentist's own convenience, financial interest, or reputation.

#370 . SJ: Professionalism

On the NDECC Situational Judgement component, if you commit a Critical Error in one station but pass the other station in that same domain, do you still fail the exam?

Quick Reference

Not necessarily. The rule requires passing at least 1 of the 2 stations in every domain, so passing the sibling station in that domain satisfies the domain requirement, provided you also pass at least 6 of 10 stations overall.

#371 . SJ: Communication and Collaboration

What three types of behaviours make up an NDECC Situational Judgement station rubric?

Quick Reference

Competent behaviours (0 points, nice to have), Minimally Competent behaviours (each worth a set point value that must sum to 1.0 to pass), and Critical Errors (automatic fail regardless of points earned elsewhere).

#372 . SJ: Communication and Collaboration

In an SJ station, what total must the point values of the minimally competent behaviours reach for the candidate to pass that station?

Quick Reference

They must sum to exactly 1.0. Falling short means the station is failed even if no critical error occurred.

#373 . SJ: Communication and Collaboration

What happens to a station's outcome if the candidate commits any listed critical error, even after earning enough points to reach 1.0?

Quick Reference

The station is an automatic fail. A critical error overrides all earned points, since critical errors carry a fixed fail value regardless of the point total.

#374 . SJ: Communication and Collaboration

What is the overall pass rule for Day 2 of the NDECC across all 10 Situational Judgement stations?

Quick Reference

The candidate must pass at least 6 of the 10 stations overall AND pass at least 1 of the 2 stations in every one of the 5 domains. Failing both stations in a single domain is an automatic overall fail even with 6 or more total passes.

#375 . SJ: Communication and Collaboration

Name the 5 domains tested across the 10 Situational Judgement stations.

Quick Reference

Patient-Centered Care, Professionalism, Communication and Collaboration, Practice and Information Management, and Health Promotion, with 2 stations in each domain.

#376 . SJ: Communication and Collaboration

What does demonstrating cultural competence mean for a dentist in a Communication and Collaboration station?

Quick Reference

Recognizing and respecting a patient's cultural, religious, or personal beliefs that may affect their care decisions, adapting communication style accordingly, and never assuming the patient's preferences based on stereotypes.

#377 . SJ: Communication and Collaboration

Give an example of a culturally competent behaviour a dentist should show with a patient from an unfamiliar background.

Quick Reference

Asking the patient directly about any beliefs, preferences, or restrictions that may affect treatment (for example around blood products, fasting, or family involvement in decisions) rather than assuming based on ethnicity or religion.

#378 . SJ: Communication and Collaboration

Why is it a risk to make assumptions about a patient's needs based on their apparent background?

Quick Reference

Assumptions can lead to a treatment plan that conflicts with the patient's actual values, damages trust, and may be scored as a lack of cultural competence or, in severe cases, a critical error for disrespectful conduct.

#379 . SJ: Communication and Collaboration

What should a dentist do when a language barrier exists between the dentist and the patient?

Quick Reference

Use a professional interpreter or translation resource when available, speak in plain simple language, confirm understanding through teach-back, and avoid relying solely on a family member for medical or consent conversations.

#380 . SJ: Communication and Collaboration

What does the sub-skill of gathering data involve in a Communication and Collaboration station?

Quick Reference

Systematically collecting relevant clinical, medical, and personal history information from the patient (or a chart, radiograph, or referral letter) to accurately understand the problem before forming a diagnosis or treatment plan.

#381 . SJ: Communication and Collaboration

Name three sources of information a dentist should gather before discussing treatment with a patient.

Quick Reference

The patient's own verbal history and chief complaint, the clinical/radiographic findings on examination, and any existing chart notes or referral documentation from another provider.

#382 . SJ: Communication and Collaboration

Why should open-ended questions be used when gathering data from a patient?

Quick Reference

Open-ended questions let the patient describe their concern in their own words, surfacing details a yes/no question would miss and helping the dentist summarize the problem accurately for the patient later.

#383 . SJ: Communication and Collaboration

What is a key behaviour when clarifying a patient's chief complaint during data gathering?

Quick Reference

Restating or summarizing what the patient said back to them to confirm accuracy, then asking follow-up questions about onset, location, duration, and severity before moving to examination findings.

#384 . SJ: Communication and Collaboration

What does maintaining patient privacy require of a dentist working in a shared or open clinic environment?

Quick Reference

Discussing sensitive findings, diagnoses, or personal history where other patients or staff cannot overhear, and keeping any visible chart, radiograph, or screen information out of view of anyone without a need to know.

#385 . SJ: Communication and Collaboration

Give an example of a privacy breach that could occur in a dental office setting.

Quick Reference

Discussing a patient's diagnosis or treatment cost loudly at the front desk within earshot of other patients, or leaving a chart open on a counter where an unrelated staff member or visitor can read it.

#386 . SJ: Communication and Collaboration

What should a dentist do before discussing a patient's diagnosis in an area where others might overhear?

Quick Reference

Move the conversation to a private operatory or consultation room, or lower voice and close the door, before discussing sensitive clinical or personal information.

#387 . SJ: Communication and Collaboration

Why must patient records be kept confidential even from other clinic staff members?

Quick Reference

Access should be limited to those with a genuine need to know for that patient's care. Sharing records or details with staff uninvolved in the patient's treatment is a privacy violation, independent of intent.

#388 . SJ: Communication and Collaboration

What key information must referral documentation to a specialist include?

Quick Reference

The patient's identifying information, the reason for referral and relevant history, clinical and radiographic findings, the specific question or service being requested of the specialist, and the referring dentist's contact information.

#389 . SJ: Communication and Collaboration

Why must a referral letter clearly state the reason for referral rather than just the tooth involved?

Quick Reference

The specialist needs to know exactly what question the referring dentist wants answered or what procedure is being requested, so care is not delayed or duplicated by an unclear or incomplete request.

#390 . SJ: Communication and Collaboration

What should a referral letter state about the urgency of the patient's condition?

Quick Reference

It should clearly flag whether the situation is urgent (for example acute infection or trauma) versus routine, so the receiving specialist can triage the appointment appropriately.

#391 . SJ: Communication and Collaboration

Name one critical error related to preparing referral documentation.

Quick Reference

Omitting information necessary for the specialist to safely treat the patient, such as leaving out a relevant medical history item or the actual reason for the referral, which prevents proper evaluation of the case.

#392 . SJ: Communication and Collaboration

What key components must be indicated on a removable partial denture (RPD) design?

Quick Reference

The major and minor connectors, the clasp assemblies (retentive and reciprocal arms), rest seats, the saddle or denture base areas, and whether each edentulous area is tooth-supported, tissue-supported, or a combination.

#393 . SJ: Communication and Collaboration

What must an RPD design specify about tooth-supported versus tissue-supported edentulous areas?

Quick Reference

The design must clearly indicate which saddle areas are bounded by teeth (tooth-supported) and which are free-end (tissue-supported), since this determines the type of rest, clasp design, and indirect retention needed.

#394 . SJ: Communication and Collaboration

Name three components typically drawn on an RPD design chart.

Quick Reference

Major connector, clasps with rest seats, and the denture base outline over each edentulous saddle area.

#395 . SJ: Communication and Collaboration

Why must a partial denture design specify the clasp type and its exact tooth location?

Quick Reference

Clasp type and placement determine retention, stability, and support, and affect the abutment tooth's health, so the laboratory needs the exact tooth and clasp design specified to fabricate the framework correctly.

#396 . SJ: Communication and Collaboration

What fields must a dental laboratory prescription include?

Quick Reference

Patient name and identifying information, tooth number(s) using FDI notation, the specific restoration or appliance requested, material and shade specified, any special instructions or design details, the date the case is required back, and the dentist's signature and licence information.

#397 . SJ: Communication and Collaboration

Why must a lab prescription always specify the shade and material to be used?

Quick Reference

Without an explicit shade and material, the laboratory cannot guarantee an esthetic or biocompatible match, and an incorrect or unspecified material choice could compromise fit, function, or patient safety.

#398 . SJ: Communication and Collaboration

What professional requirement applies to signing a dental lab prescription?

Quick Reference

The prescribing dentist must sign and date the prescription, taking responsibility for its accuracy, since the laboratory technician fabricates strictly according to the written prescription and cannot diagnose or alter the treatment plan.

#399 . SJ: Communication and Collaboration

What should a lab prescription state regarding the return date of the case?

Quick Reference

It should specify the date or timeframe the finished appliance or restoration is needed back, so the laboratory can schedule fabrication and the dentist can plan the patient's next appointment accordingly.

#400 . SJ: Communication and Collaboration

What are the key elements of an effective treatment discussion station, per NDEB's worked example?

Quick Reference

Summarizing the problem with the tooth, presenting more than one complete treatment option, explaining the advantages and disadvantages of each option and the preferred choice, verifying the patient understands, and communicating in an organized, effective manner.

#401 . SJ: Communication and Collaboration

Why must a dentist provide more than one treatment option during a treatment discussion?

Quick Reference

Presenting a single option does not respect patient autonomy or allow true informed decision-making. Giving multiple complete options, with pros and cons of each, is one of the minimally competent behaviours required to reach the passing point total.

#402 . SJ: Communication and Collaboration

What behaviour confirms a patient actually understood the treatment discussion, not just heard it?

Quick Reference

Asking the patient to restate the plan in their own words, or directly checking comprehension (teach-back), rather than simply asking if they have questions.

#403 . SJ: Communication and Collaboration

Name one 0-point (competent, nice-to-have) behaviour listed in NDEB's treatment discussion worked example.

Quick Reference

Answering any questions the patient has, giving the patient an opportunity to ask questions, discussing the patient's special circumstances if applicable, or discussing fees. These are expected but do not by themselves determine pass or fail.

#404 . SJ: Communication and Collaboration

What are the essential elements of valid informed consent?

Quick Reference

Disclosure of the diagnosis and proposed treatment, the risks and benefits, reasonable alternatives (including no treatment), confirmation the patient has the capacity to decide, and voluntary agreement free of coercion, with an opportunity for the patient to ask questions.

#405 . SJ: Communication and Collaboration

Why must risks, benefits, and alternatives all be discussed for consent to be considered valid?

Quick Reference

Consent is only informed if the patient understands what they are agreeing to, what could go wrong, and what other reasonable choices exist, including declining treatment. Omitting any of these makes the consent incomplete and not truly informed.

#406 . SJ: Communication and Collaboration

What can make informed consent invalid even if the patient has signed a written consent form?

Quick Reference

If the patient was not actually given an understandable explanation of the risks, benefits, and alternatives, or did not have the opportunity to ask questions, the signature alone does not make the consent valid.

#407 . SJ: Communication and Collaboration

What must a dentist confirm with the patient before proceeding with treatment under informed consent?

Quick Reference

That the patient understands the diagnosis and proposed treatment, has had their questions answered, and voluntarily agrees to proceed, verified by checking comprehension rather than assuming silence means agreement.

#408 . SJ: Communication and Collaboration

What communication behaviour is expected when a dentist interacts with clinical or auxiliary staff?

Quick Reference

Communicating instructions clearly, respectfully, and in an organized manner, listening to staff observations or concerns, and collaborating as a team rather than issuing directions without explanation.

#409 . SJ: Communication and Collaboration

Give an example of a critical error in a staff interaction station.

Quick Reference

Dismissing or ignoring a staff member's safety concern (for example about a contaminated instrument or a patient reaction) and proceeding with treatment anyway, jeopardizing patient or operator safety.

#410 . SJ: Communication and Collaboration

Why is respectful collaboration with dental assistants and hygienists important for patient safety?

Quick Reference

Staff often notice details the dentist may miss (medical alerts, patient distress, contamination). Poor communication or dismissiveness can cause important safety information to be missed or ignored.

#411 . SJ: Communication and Collaboration

What should a dentist do if a staff member raises a safety concern during a procedure?

Quick Reference

Stop and listen to the concern, evaluate it before proceeding, and address it appropriately rather than overriding it without discussion, since dismissing a valid safety concern can constitute a critical error.

#412 . SJ: Communication and Collaboration

What must post-operative instructions given to a patient include?

Quick Reference

Expected symptoms and their normal duration, pain and swelling management, dietary and activity restrictions, oral hygiene modifications, signs of a complication to watch for, and when and how to contact the office if a problem arises.

#413 . SJ: Communication and Collaboration

Why should post-operative instructions be given both verbally and in writing?

Quick Reference

Verbal delivery allows immediate clarification and confirms understanding in the moment, while written instructions give the patient an accurate reference after anesthesia or stress may have reduced their recall.

#414 . SJ: Communication and Collaboration

What would count as a critical error related to giving post-operative instructions?

Quick Reference

Failing to give the patient any instructions at all, or omitting a safety-critical warning sign (such as spreading infection or uncontrolled bleeding) that the patient needs to know to seek urgent care.

#415 . SJ: Communication and Collaboration

What should post-operative instructions include regarding an emergency or after-hours contact?

Quick Reference

Clear information on how to reach the dentist or on-call coverage if a complication such as severe pain, swelling, bleeding, or fever develops outside regular office hours.

#416 . SJ: Practice and Information Management

How many Situational Judgement stations are on Day 2 of the NDECC, and how many domains do they cover?

Quick Reference

10 stations total, 2 stations in each of 5 domains: Patient-Centered Care, Professionalism, Communication and Collaboration, Practice and Information Management, and Health Promotion.

#417 . SJ: Practice and Information Management

What is the overall pass rule for the Situational Judgement component?

Quick Reference

You must pass at least 6 of the 10 stations, AND pass at least 1 of the 2 stations in every domain. Failing both stations in one domain is an automatic overall fail, even with 6 or more total passes.

#418 . SJ: Practice and Information Management

How is each Situational Judgement station scored?

Quick Reference

Pass/Fail via a weighted-behavior checklist: Competent behaviors are worth 0 points (nice to have), Minimally Competent behaviors each carry a point value that must sum to 1.0 to pass, and any Critical Error is an automatic fail regardless of points earned.

#419 . SJ: Practice and Information Management

In the SJ rubric, what does a 0-point Competent behavior mean for pass/fail?

Quick Reference

It does not affect pass/fail on its own. It is a nice-to-have behavior that shows optimal performance but earns no required points.

#420 . SJ: Practice and Information Management

In the SJ rubric, what must the Minimally Competent point-bearing behaviors do to pass a station?

Quick Reference

Their point values must sum to exactly 1.0. For example, five behaviors each worth 0.20 points that together reach 1.0.

#421 . SJ: Practice and Information Management

What happens at an SJ station if a candidate commits a listed Critical Error, even after earning 1.0 in points?

Quick Reference

The station is an automatic fail. A Critical Error overrides any points already earned toward the 1.0 threshold.

#422 . SJ: Practice and Information Management

Name the 3 sub-skills that fall under the Practice and Information Management domain.

Quick Reference

Maintaining and managing patient records, following protocols, and making evidence-based decisions.

#423 . SJ: Practice and Information Management

Roughly how long is each SJ station, including reading and transition time?

Quick Reference

Approximately 20 to 24 minutes per station, based on the half-day for 10 stations. NDEB does not publish exact per-station minutes, so this is a disclosed estimate, not an official figure.

#424 . SJ: Practice and Information Management

What must a complete patient chart entry document, at minimum, after a clinical visit?

Quick Reference

The date of service, tooth and surface(s) treated using FDI notation, the diagnosis or reason for treatment, the procedure performed, materials and anesthetic used, any complications, and post-op instructions given to the patient.

#425 . SJ: Practice and Information Management

Why does a chart entry need the specific tooth number in FDI notation rather than a vague description like left upper molar?

Quick Reference

A precise tooth number such as tooth 2.6 prevents ambiguity for future providers, supports continuity of care, and protects the dentist if the record is ever reviewed for a complaint or audit.

#426 . SJ: Practice and Information Management

What information about consent should a chart entry always capture before a procedure?

Quick Reference

That informed consent was obtained, including that risks, benefits, and alternatives (including no treatment) were discussed and the patient had the opportunity to ask questions.

#427 . SJ: Practice and Information Management

A patient calls the next day reporting pain after an extraction. What must the record of that call include?

Quick Reference

The date and time of the call, the patient's reported symptoms, the advice given, and any follow-up appointment scheduled. Undocumented advice is treated as advice never given.

#428 . SJ: Practice and Information Management

Why should a chart correction never simply erase or delete the original entry?

Quick Reference

The original entry must remain legible with the correction added and dated alongside it, so the record shows a clear, honest audit trail rather than an alteration that could look like a cover-up.

#429 . SJ: Practice and Information Management

What should happen when a patient declines a recommended treatment?

Quick Reference

The refusal, the risks of not treating that were explained, and the patient's decision must all be documented in the chart, protecting both patient autonomy and the practitioner.

#430 . SJ: Practice and Information Management

How should a referral to a specialist be documented in practice management terms?

Quick Reference

The chart should note the reason for referral, the specialist's name, the information sent with the referral, and confirmation the patient understood why the referral was made.

#431 . SJ: Practice and Information Management

What is the general standard for how long dental patient records must be retained after the last entry, in most Canadian provinces?

Quick Reference

Commonly cited as 10 years or more from the last entry, though the exact requirement varies by province and by whether the patient was a minor at the time of treatment, so always confirm the specific rule of the relevant provincial regulatory college.

#432 . SJ: Practice and Information Management

If a patient requests a copy of their own dental record, what is the general obligation?

Quick Reference

The practice must provide access to the record (or a copy) within a reasonable time, since the patient generally owns the information even though the physical chart belongs to the practice.

#433 . SJ: Practice and Information Management

Name three hand hygiene moments that infection control protocols require in a dental operatory.

Quick Reference

Before gloving, after removing gloves, and whenever moving between patients. Hand hygiene not performed, or not performed properly, is one of the 15 named IC&S; errors.

#434 . SJ: Practice and Information Management

What personal protective equipment must be worn correctly throughout a clinical procedure per IC&S; protocol?

Quick Reference

A mask worn correctly and eye protection worn correctly. Either one missing or worn incorrectly is a named IC&S; error.

#435 . SJ: Practice and Information Management

Under infection control protocol, what is prohibited once treatment gloves are on?

Quick Reference

Touching your own face, hair, or mask, adjusting hearing aid batteries, or touching any other non-sterile item with treatment gloves. Loupes are the one explicit exception, they may be touched or adjusted with gloves on.

#436 . SJ: Practice and Information Management

How many total IC&S; errors are permitted across an entire clinical day, and what happens on the next one?

Quick Reference

A maximum of 3 errors are permitted; a 4th IC&S; error of any kind is an automatic fail for the day.

#437 . SJ: Practice and Information Management

What must be used with a high-speed handpiece to avoid an IC&S; error?

Quick Reference

Adequate cooling water. No or insufficient cooling water with a high-speed handpiece is a named IC&S; error, since it risks thermal damage to the pulp.

#438 . SJ: Practice and Information Management

What is required for sharps and needles under infection control protocol?

Quick Reference

Acceptable handling and disposal into a designated sharps container. Unacceptable handling or disposal of amalgam, sharps, or needles is a named IC&S; error.

#439 . SJ: Practice and Information Management

In everyday practice, what basic protocol step prevents cross-contamination between operatories?

Quick Reference

Using single-use barriers or properly disinfecting/sterilizing surfaces and instruments between every patient, and never reusing a contaminated instrument or material.

#440 . SJ: Practice and Information Management

What is the first step of evidence-based decision making when facing a clinical question?

Quick Reference

Formulate a clear, focused clinical question, then search for the best available current research evidence relevant to that specific question.

#441 . SJ: Practice and Information Management

Name the three components that evidence-based dentistry weighs together, according to the standard framework.

Quick Reference

The best available scientific evidence, the clinician's own clinical expertise, and the patient's individual needs, values, and preferences.

#442 . SJ: Practice and Information Management

Why is patient preference still part of an evidence-based decision even when strong research supports one option?

Quick Reference

Because a treatment plan must be acceptable to the patient to succeed; ignoring the patient's values and circumstances risks non-adherence even if the evidence for the option is strong.

#443 . SJ: Practice and Information Management

A patient asks why you recommend a stainless steel crown over a large filling for a badly broken-down primary molar. What evidence-based reasoning supports your answer?

Quick Reference

Research shows stainless steel crowns have a substantially higher long-term success rate than large multi-surface restorations on primary molars, reducing the risk of failure and the need for retreatment.

#444 . SJ: Practice and Information Management

When a treatment protocol conflicts with a patient's specific medical history, what should guide the final decision?

Quick Reference

Clinical judgement applied to that patient's individual circumstances, informed by the evidence, rather than following the standard protocol rigidly regardless of context.

#445 . SJ: Practice and Information Management

Why should a dentist periodically review and update their own clinical protocols?

Quick Reference

Because evidence-based practice requires protocols to reflect current best evidence, not outdated training; protocols that are never reviewed risk falling behind updated clinical guidelines.

#446 . SJ: Practice and Information Management

What documentation is expected when a dentist deviates from a written office protocol for a specific patient?

Quick Reference

The chart should note the deviation and the clinical reasoning behind it, so the record clearly shows an informed decision rather than an unexplained inconsistency.

#447 . SJ: Practice and Information Management

In a Practice and Information Management SJ station, what kind of point-bearing behavior would likely appear for a record-keeping scenario?

Quick Reference

Behaviors such as accurately documenting the relevant clinical findings, recording informed consent, and noting the treatment provided, each worth a point value summing to 1.0.

#448 . SJ: Practice and Information Management

What is a likely Critical Error in a Practice and Information Management station involving records?

Quick Reference

Falsifying a record, backdating an entry, or altering a chart to conceal an error would be treated as a critical error resulting in automatic failure of that station.

#449 . SJ: Practice and Information Management

What administrative step should follow discovering a lab prescription error before the case returns from the lab?

Quick Reference

Contact the lab promptly to correct the prescription, and document the error and the correction made in the patient record.

#450 . SJ: Practice and Information Management

What basic charting habit protects a dentist if a patient later disputes what was discussed at a visit?

Quick Reference

Recording, at the time of the visit, exactly what was discussed, including options presented and questions answered, rather than relying on memory later.

#451 . SJ: Practice and Information Management

Why must radiographs and other diagnostic images be kept as part of the patient record, with dates?

Quick Reference

Dated images allow comparison over time to track disease progression or treatment outcomes, and they support the evidence behind past diagnoses and decisions if the record is reviewed later.

#452 . SJ: Practice and Information Management

What protocol applies to storing patient information electronically in most Canadian jurisdictions?

Quick Reference

Electronic records must be kept secure and confidential, with access limited to authorized staff and appropriate safeguards against unauthorized disclosure, consistent with privacy legislation.

#453 . SJ: Practice and Information Management

When following an infection control protocol, what should happen if a barrier or instrument becomes contaminated mid-procedure?

Quick Reference

It must be replaced or re-sterilized immediately before continuing; continuing with a known-contaminated item is a named IC&S; error.

#454 . SJ: Practice and Information Management

What is the practical difference between following a protocol and making an evidence-based decision?

Quick Reference

Following a protocol means applying an established standard procedure consistently; making an evidence-based decision means actively weighing current research, clinical expertise, and patient factors, sometimes justifying a deviation from the standard protocol.

#455 . SJ: Practice and Information Management

Why does NDEB test Practice and Information Management as its own Situational Judgement domain rather than folding it into Professionalism?

Quick Reference

Because record-keeping, protocol adherence, and evidence-based decision making are distinct, practical competencies tied to safe practice management, separate from ethical judgement or communication skills tested elsewhere.

#456 . SJ: Health Promotion

How many stations make up the Situational Judgement (SJ) component of the NDECC?

Quick Reference

10 stations, 2 stations in each of 5 domains.

#457 . SJ: Health Promotion

List the 5 domains tested in the SJ component.

Quick Reference

Patient-Centered Care, Professionalism, Communication and Collaboration, Practice and Information Management, and Health Promotion.

#458 . SJ: Health Promotion

What is the overall pass rule for the SJ component?

Quick Reference

The candidate must pass at least 6 of 10 stations AND at least 1 of the 2 stations in each domain; failing both stations in one domain is an overall fail.

#459 . SJ: Health Promotion

How is each individual SJ station graded?

Quick Reference

Pass or fail, using a weighted behaviour checklist specific to that station's scenario.

#460 . SJ: Health Promotion

**In the SJ scoring rubric, what are
Competent behaviours worth?**

Quick Reference

0 points each. They are nice-to-have behaviours that do not by themselves determine pass or fail.

#461 . SJ: Health Promotion

In the SJ scoring rubric, how must the Minimally Competent behaviour points total for a station to pass?

Quick Reference

They must sum to exactly 1.0.

#462 . SJ: Health Promotion

What happens if a candidate commits a listed Critical Error during an SJ station?

Quick Reference

Automatic fail for that station, regardless of any other points already earned.

#463 . SJ: Health Promotion

About how long is each SJ station timed?

Quick Reference

About 20 to 24 minutes including reading and transition time, a disclosed estimate; NDEB does not officially publish exact per-station minutes.

#464 . SJ: Health Promotion

What is the maximum number of Infection Control and Safety (IC&S;) errors a candidate may commit across the whole Clinical Skills day?

Quick Reference

3 errors; a 4th error is an automatic fail.

#465 . SJ: Health Promotion

Name two IC&S; errors related to personal protective equipment.

Quick Reference

Mask not worn or not worn correctly, and eye protection not worn or not worn correctly.

#466 . SJ: Health Promotion

Is touching or adjusting loupes with gloves on considered an IC&S; error?

Quick Reference

No, this is explicitly permitted and is not counted as an error.

#467 . SJ: Health Promotion

Give an example of an IC&S; error related to instrument or material handling.

Quick Reference

Use of contaminated instruments or materials, or unacceptable handling or disposal of amalgam, sharps, or needles.

#468 . SJ: Health Promotion

Name three patient-level factors that increase caries risk.

Quick Reference

Frequent sugar or fermentable carbohydrate intake, poor oral hygiene, and low salivary flow (xerostomia).

#469 . SJ: Health Promotion

How does xerostomia increase caries risk?

Quick Reference

Reduced saliva means less buffering of plaque acids and less clearance of food debris and bacteria, letting demineralization progress unchecked.

#470 . SJ: Health Promotion

What socioeconomic factor is one of the strongest predictors of high caries risk in a new patient?

Quick Reference

Low household income or lack of dental insurance, which limits access to preventive and restorative care.

#471 . SJ: Health Promotion

Why are patients with fixed orthodontic appliances considered higher caries risk?

Quick Reference

Brackets and wires create plaque-retentive areas that are harder to clean, increasing demineralization risk around the brackets.

#472 . SJ: Health Promotion

Why is a history of previous caries activity an important caries risk factor?

Quick Reference

Past caries experience is one of the single strongest predictors of future caries, reflecting ongoing behavioural and biological risk.

#473 . SJ: Health Promotion

What medical or behavioural factor common in older adults raises caries risk through dry mouth?

Quick Reference

Polypharmacy; many prescription medications list xerostomia as a side effect, reducing salivary protection.

#474 . SJ: Health Promotion

A patient with visible plaque, frequent snacking, and no fluoride exposure would be classified at what caries risk level?

Quick Reference

High caries risk.

#475 . SJ: Health Promotion

Define social determinants of health (SDOH) in the dental context.

Quick Reference

The social, economic, and environmental conditions, such as income, education, housing, and employment, that shape a patient's ability to access and maintain oral health.

#476 . SJ: Health Promotion

How can low health literacy affect a patient's oral health outcomes?

Quick Reference

The patient may not understand home care instructions, follow through on referrals, or recognize warning signs, leading to delayed treatment and worse outcomes.

#477 . SJ: Health Promotion

Why might a rural patient have worse access to dental care, as an SDOH issue?

Quick Reference

Fewer dental providers, longer travel distances, and limited transportation reduce the likelihood of regular preventive visits.

#478 . SJ: Health Promotion

How does employment status act as a social determinant of oral health?

Quick Reference

Lack of employer dental benefits or the inability to take time off work reduces the likelihood of seeking timely dental treatment.

#479 . SJ: Health Promotion

What is a culturally competent approach to discussing oral health with a newcomer patient?

Quick Reference

Ask about the patient's own understanding and beliefs first, use plain language or an interpreter as needed, and avoid assumptions based on background.

#480 . SJ: Health Promotion

Why should a dentist screen for food insecurity when assessing caries risk?

Quick Reference

Reliance on inexpensive, high-carbohydrate foods increases caries risk, and food insecurity is a social determinant that should guide individualized prevention counselling.

#481 . SJ: Health Promotion

Name two practical ways a dentist can promote access to care for an uninsured patient.

Quick Reference

Discuss a phased or lower-cost treatment plan, and inform the patient of public dental programs they may qualify for.

#482 . SJ: Health Promotion

What should a dentist do if a patient cannot afford the ideal treatment plan?

Quick Reference

Present a reasonable alternative option within the patient's means rather than refusing to treat, and document the discussion.

#483 . SJ: Health Promotion

How can a dental office reduce barriers to care for patients with mobility limitations?

Quick Reference

Offer accessible scheduling, wheelchair-accessible operatories, and flexible appointment times.

#484 . SJ: Health Promotion

What is one role a dentist can play in connecting a patient to public dental benefit programs?

Quick Reference

Provide information on eligibility for provincial or territorial public dental programs and assist with referral or application steps.

#485 . SJ: Health Promotion

Why is continuity of care important for promoting access, especially for children?

Quick Reference

Regular recall visits catch problems early and build trust, reducing the likelihood of emergency-only, reactive dental visits.

#486 . SJ: Health Promotion

What concentration of fluoride varnish is most commonly used in preventive care?

Quick Reference

5% sodium fluoride, equal to 22,600 ppm fluoride.

#487 . SJ: Health Promotion

List two indications for applying fluoride varnish.

Quick Reference

A high caries risk patient, or a patient with active white spot lesions or early demineralization.

#488 . SJ: Health Promotion

How often is fluoride varnish typically reapplied for a high caries risk patient?

Quick Reference

Every 3 to 6 months, depending on the patient's individual risk level.

#489 . SJ: Health Promotion

What is the primary indication for placing a pit and fissure sealant?

Quick Reference

A newly erupted permanent molar or premolar with deep pits and fissures at risk of caries, placed before decay has started.

#490 . SJ: Health Promotion

On which teeth are sealants most commonly and effectively placed?

Quick Reference

Permanent first and second molars, such as tooth 3.6 or tooth 4.6, soon after eruption.

#491 . SJ: Health Promotion

Should a sealant be placed over a tooth with an existing frank cavitated lesion?

Quick Reference

No. A sealant is only appropriate for sound or early non-cavitated enamel; a cavitated lesion needs restorative treatment.

#492 . SJ: Health Promotion

What type of mouthguard offers the best protection and fit for a patient in contact sports?

Quick Reference

A custom-fabricated mouthguard made on a model of the patient's own dentition.

#493 . SJ: Health Promotion

Name the three general categories of athletic mouthguards.

Quick Reference

Stock (ready-made), boil-and-bite (mouth-formed), and custom-fabricated. A custom-fabricated mouthguard, made from a dental impression, generally offers the best fit, retention, and comfort of the three.

#494 . SJ: Health Promotion

As part of injury prevention, why should a dentist recommend a mouthguard for a patient who plays contact sports?

Quick Reference

Mouthguards absorb and distribute impact forces, reducing the risk of tooth avulsion, fracture, and jaw injury.

#495 . SJ: Health Promotion

What patient population should be counselled on mouthguard-type protection even outside organized sports?

Quick Reference

Patients who grind or clench (bruxism) may benefit from a night guard, distinct from an athletic mouthguard, to prevent tooth wear or fracture.

#496 . SJ: Health Promotion

What are the two most significant modifiable risk factors for oral cancer that should be part of every screening discussion?

Quick Reference

Tobacco use, smoked or smokeless, and heavy alcohol consumption, especially the two combined.

#497 . SJ: Health Promotion

What visual and palpation steps make up a basic oral cancer screening?

Quick Reference

Extraoral inspection and palpation of the head, neck, and lymph nodes, plus intraoral inspection of the lips, cheeks, tongue including the lateral borders, floor of mouth, and palate for ulcers, lesions, or masses that do not resolve.

#498 . SJ: Health Promotion

What clinical feature of an oral lesion should prompt urgent referral for possible malignancy?

Quick Reference

A non-healing ulcer or lesion persisting longer than 2 weeks, especially if indurated, fixed, or associated with unexplained numbness.

#499 . SJ: Health Promotion

What does patient advocacy mean in the context of the Health Promotion domain?

Quick Reference

Acting in the patient's best interest by helping them access needed care, referrals, or resources, and speaking up on their behalf within the healthcare system.

#500 . SJ: Health Promotion

Give an example of patient advocacy for a child with early childhood caries whose parent faces financial barriers.

Quick Reference

Referring the family to a public dental program, explaining prevention clearly, and prioritizing urgent treatment needs within the family's means.